



Nursing process

Multiple Choice Questions

1. **The nursing process is best defined as:**
 - ☒ **A. A systematic method of providing individualized care.**
 - ☐ B. A list of physician's orders to be followed.
 - ☐ C. A medical way to cure diseases.
 - ☐ D. A static one-time assessment.
2. **Which phase involves collecting, organizing, and validating data?**
 - ☒ **A. Assessment.**
 - ☐ B. Diagnosis.
 - ☐ C. Planning.
 - ☐ D. Implementation.
3. **"Subjective data" are also known as:**
 - ☐ A. Signs.
 - ☒ **B. Symptoms.**
 - ☐ C. Lab results.
 - ☐ D. Physical findings.
4. **"Objective data" are also known as:**
 - ☒ **A. Signs.**
 - ☐ B. Symptoms.
 - ☐ C. Client's feelings.
 - ☐ D. Patient's worries.
5. **In which step does the nurse identify clients' health problems?**
 - ☐ A. Evaluation.
 - ☐ B. Implementation.
 - ☒ **C. Diagnosis.**
 - ☐ D. Planning.
6. **Prioritizing nursing diagnoses is usually based on:**
 - ☐ A. The nurse's schedule.
 - ☐ B. Hospital policy.
 - ☒ **C. Maslow's hierarchy of needs.**
 - ☐ D. Alphabetical order.
7. **Which of the following is an "Independent Intervention"?**
 - ☒ **A. Teaching a client about their medication.**
 - ☐ B. Giving an injection ordered by a doctor.
 - ☐ C. Starting an IV line under supervision.
 - ☐ D. Performing surgery.
8. **The phase that consists of "doing and documenting" is:**
 - ☐ A. Planning.
 - ☒ **B. Implementation.**
 - ☐ C. Diagnosis.
 - ☐ D. Assessment.
9. **If a goal is not met during the evaluation phase, the nurse should:**
 - ☒ **A. Modify the care plan.**
 - ☐ B. Terminate the care plan.
 - ☐ C. Ignore the results.
 - ☐ D. Stop the nursing process.



10. Which characteristic means the steps of the nursing process are interrelated?
- ☒ A. Cyclic and dynamic.
 - ☐ B. Client-centered.
 - ☐ C. Goal-oriented.
 - ☐ D. Universally applicable.
11. A "Nursing Diagnosis" focuses on which of the following?
- ☐ A. The disease process itself.
 - ☒ B. Human responses to health conditions.
 - ☐ C. Pathophysiological changes.
 - ☐ D. Surgical requirements.
12. When a nurse collects a blood pressure reading of 150/90, this is:
- ☒ A. Objective data.
 - ☐ B. Subjective data.
 - ☐ C. A symptom.
 - ☐ D. Secondary data.
13. The "Planning" phase includes which of the following activities?
- ☐ A. Collecting a database.
 - ☐ B. Implementing nursing actions.
 - ☒ C. Setting priorities and goals.
 - ☐ D. Documenting care provided.
14. "Discharge Planning" should ideally begin:
- ☒ A. At the time of admission.
 - ☐ B. The day before the client leaves.
 - ☐ C. After the surgery is finished.
 - ☐ D. When the patient is stable.
15. Which characteristic of the nursing process ensures it can be used in any healthcare setting?
- ☐ A. Client-centered.
 - ☐ B. Goal-oriented.
 - ☒ C. Universal applicability.
 - ☐ D. Collaborative.
16. Validation of data means:
- ☐ A. Documenting the data.
 - ☒ B. Double-checking that the data is accurate and factual.
 - ☐ C. Organizing data into clusters.
 - ☐ D. Asking the doctor to sign the data.
17. Collaborative interventions are also known as:
- ☐ A. Independent interventions.
 - ☐ B. Dependent interventions.
 - ☒ C. Interdependent interventions.
 - ☐ D. Medical interventions.
18. Which type of planning is performed immediately after the initial assessment of the patient?
- A- Ongoing Planning
 - B- Discharge Planning
 - ☒ C- Initial Planning
 - D- Strategic Planning



19. Interventions performed by the nurse in collaboration with other health team members are called:
- A- Independent Interventions
 - B- Dependent Interventions
 - ☒ C- Collaborative Interventions
 - D- Basic Interventions
20. Which of the following is an example of an "Independent Nursing Intervention"?
- A- Administering medication based on a physician's order.
 - ☒ B- Patient teaching (e.g., teaching a client how to change a dressing).
 - C- Ordering blood tests for the patient.
 - D- Prescribing a specific diet requested by the physician.
21. The phase in the nursing process that involves "analytical thinking and identifying health strengths and problems" is:
- A- Assessment
 - ☒ B- Diagnosis
 - C- Implementation
 - D- Planning
22. What is the first step of the nursing process?
- A. Planning
 - B. Assessment**
 - C. Implementation
 - D. Diagnosis
23. Which organization is responsible for the standard classification of nursing diagnoses?
- A. WHO
 - B. ICN
 - C. NANDA**
 - D. ANA
24. Data collected from a source other than the patient (e.g., family or medical records) is called:
- A. Primary data
 - B. Subjective data
 - C. Secondary data**
 - D. Objective data
25. Which statement best describes the "Evaluation" phase of the nursing process?
- A. It is the final step and occurs only once.
 - B. It is a continuous process to judge the effectiveness of care.**
 - C. It focuses only on the nurse's performance.
 - D. It is used primarily to identify new medical diseases.
26. What is the main difference between a Nursing Diagnosis and a Medical Diagnosis?
- A. Nursing diagnosis focuses on human responses, while medical diagnosis focuses on the disease process.**
 - B. Medical diagnosis changes daily, while nursing diagnosis remains permanent.
 - C. Only physicians can write a nursing diagnosis.
 - D. Nursing diagnosis is only used in emergency situations.



27. Which of the following describes a "Risk Nursing Diagnosis"?
- A. A problem that is currently present and has signs/symptoms.
 - B. A problem does not yet exist but has a high probability of developing.**
 - C. A diagnosis that focuses only on healthy patients.
 - D. A medical condition that requires surgery.
28. A nurse is caring for a patient who is post-surgery. The nurse notices the patient is grimacing and holding their abdomen. The nurse records: "Patient is holding abdomen and grimacing." This is an example of:
- A. Subjective data
 - B. Objective data**
 - C. Secondary data
 - D. Interpretation
29. The nurse establishes a goal: "The patient will walk 50 feet with a walker by the end of the shift." This activity occurs during which phase?
- A. Assessment
 - B. Diagnosis
 - C. Planning**
 - D. Evaluation
30. A nurse carries out a physician's order to administer 500mg of an antibiotic. This is classified as which type of intervention?
- A. Independent intervention
 - B. Dependent intervention**
 - C. Collaborative intervention
 - D. Autonomous intervention
31. After evaluating a patient, the nurse finds that the goal "Patient will lose 2 lbs in a week" was not met. What should the nurse's next action be?
- A. Terminate the care plan immediately.
 - B. Re-assess the patient and modify the care plan.**
 - C. Tell the patient they failed the treatment.
 - D. Documentation only, without changing the plan.
32. A nurse is performing an assessment on a patient admitted 5 minutes ago with severe chest pain and difficulty breathing. Which type of assessment is most appropriate?
- A. Initial assessment
 - B. Problem-focused assessment
 - C. Emergency assessment**
 - D. Time-lapsed assessment
33. In the nursing diagnosis "Constipation related to insufficient fluid intake as manifested by hard, dry stool," the phrase "insufficient fluid intake" is the:
- A. Problem (Diagnostic Label)
 - B. Etiology (Related factors)**
 - C. Signs and Symptoms (Defining characteristics)
 - D. Outcome identification



34. A nurse helps a patient perform deep breathing exercises to prevent pneumonia. This is an example of which nursing process phase?
- A. Assessment
 - B. Planning
 - C. Implementation**
 - D. Evaluation
35. A nurse is caring for a patient with a nursing diagnosis of "Impaired Gas Exchange." During the evaluation, the nurse finds the oxygen saturation is 94% (Goal was >95%), but the patient's respiratory rate has decreased from 30 to 22 breaths/minute. What is the most "intelligent" nursing action?
- A. Terminate the care plan because the patient is better.
 - B. Re-assess and modify the plan, as the goal is partially met.**
 - C. Ignore the oxygen saturation and only focus on the respiratory rate.
 - D. Immediately call the physician to report a failure in nursing care.
36. Which of the following scenarios demonstrates the nurse using "Critical Thinking" during the Implementation phase?
- A. Administering a medication exactly at the time ordered without checking the patient's condition.
 - B. Following the care plan strictly even if the patient's status has changed.
 - C. Stopping a scheduled ambulation (walking) exercise because the patient is suddenly feeling dizzy.**
 - D. Checking the patient's ID band before giving a treatment.
37. If a nurse uses a "Maslow's Hierarchy of Needs" framework to prioritize nursing diagnoses, which of the following should be addressed FIRST?
- A. Risk for Loneliness related to social isolation.
 - B. Ineffective Airway Clearance related to excessive secretions.**
 - C. Readiness for Enhanced Knowledge related to diabetes management.
 - D. Disturbed Body Image related to surgical scar.
38. When comparing a Nursing Diagnosis of "Acute Pain" with a Medical Diagnosis of "Appendicitis," which statement shows the best understanding?
- A. They are the same thing and can be used interchangeably.
 - B. The Nursing Diagnosis focuses on the patient's experience of pain and its effect on function.**
 - C. The nurse must wait for the Medical Diagnosis before treating the pain.
 - D. The Medical Diagnosis is only for surgeons, and the Nursing Diagnosis is only for students.
39. In the nursing diagnosis: "Impaired Skin Integrity related to prolonged bed rest as evidenced by a 2cm reddened area on the sacrum." What is the Etiology?
- A. Impaired Skin Integrity
 - B. Prolonged bed rest**
 - C. 2cm reddened area
 - D. Sacrum location



40. Which part of the following diagnosis represents the "Defining Characteristics" (Signs/Symptoms)? "Ineffective Airway Clearance related to excessive secretions as evidenced by adventitious breath sounds and dyspnea."
- A. Ineffective Airway Clearance
 - B. Excessive secretions
 - C. Adventitious breath sounds and dyspnea**
 - D. Related to secretions
41. In the nursing diagnosis: "Acute Pain related to physical trauma as manifested by patient rating pain 9/10 on scale." The "Problem" or Diagnostic Label is:
- A. Physical trauma
 - B. Patient rating pain 9/10
 - C. Acute Pain**
 - D. Pain scale
42. A nurse is writing a diagnosis for a patient who has not moved for 3 days and is at risk for bedsores, but currently has perfectly clear skin. Which diagnosis is written correctly?
- A. Impaired Skin Integrity related to immobility as evidenced by redness.
 - B. Risk for Impaired Skin Integrity related to immobility.**
 - C. Actual Bedsores related to 3 days of no movement.
 - D. Possible Skin Integrity related to bed rest.
43. Which of the following is a correctly structured "Actual" Nursing Diagnosis (3-part statement)?
- A. Readiness for enhanced nutrition.
 - B. Risk for falls related to dizziness.
 - C. Anxiety related to upcoming surgery as evidenced by increased heart rate and restlessness.**
 - D. Pneumonia related to bacterial infection as evidenced by cough.
44. Data: Patient says "I feel very weak," Heart Rate 110 bpm, patient unable to walk to the bathroom without help. From the patient card above, if we formulate the diagnosis "Activity Intolerance related to imbalance between oxygen supply and demand as evidenced by heart rate 110 bpm and verbal report of weakness," what does "Activity Intolerance" represent?
- A. The Etiology (The Cause)
 - B. The Diagnostic Label (The Problem)**
 - C. The Signs (The Evidence)
 - D. The Medical Diagnosis
45. "Cluster the following data": (Dry skin, Patient states "I am thirsty," Urine output 200ml/24hr). Which Nursing Diagnosis matches this intelligent grouping?
- A. Deficient Fluid Volume**
 - B. Impaired Tissue Integrity
 - C. Excess Fluid Volume
 - D. Acute Pain



46. Why is "Chronic Obstructive Pulmonary Disease (COPD)" NOT a nursing diagnosis?
- A. Because it describes a human response.
 - B. Because it is a medical condition that remains constant and describes a specific disease process.**
 - C. Because nurses are not allowed to treat patients with lung problems.
 - D. Because it only has one part.
47. When a nurse modifies a nursing diagnosis after seeing the patient's symptoms have improved, which characteristic of the Nursing Process is being applied?
- A. Static nature
 - B. Cyclic and Dynamic nature**
 - C. Physician-centered
 - D. Randomization
48. Which type of assessment is performed to establish a complete database for problem identification and future comparison?
- A. Problem-focused assessment.
 - B. Emergency assessment.
 - C. Time-lapsed assessment.
 - D. Initial assessment.**
49. What are the three components of a NANDA nursing diagnosis (PES format)?
- A. Problem, Etiology, Symptoms.**
 - B. Planning, Evaluation, Signs.
 - C. Physical, Emotional, Social.
 - D. Primary, Evidence, Secondary.
50. Which statement best explains the "Dynamic" characteristic of the nursing process?
- A. Each phase of the nursing process is separate and does not affect the others.
 - B. The phases are closely interrelated and affect each other. (Correct Answer)**
 - C. The process is a fixed set of rules that never changes.
 - D. It is a process used only for surgical patients.
51. A nurse is caring for a patient who is struggling to breathe. The nurse immediately starts oxygen therapy before completing a full head-to-toe exam. Which type of assessment is being used?
- A. Initial assessment.
 - B. Time-lapsed assessment.
 - C. Emergency assessment. (Correct Answer)**
 - D. Problem-focused assessment.
52. A nurse administers a prescribed medication to a patient. This is an example of which type of intervention?
- A. Independent intervention.
 - B. Dependent intervention.**
 - C. Collaborative intervention.
 - D. Self-care intervention.



53. Analyze this nursing diagnosis: "Risk for Infection related to surgical incision." Why is this considered a "Risk" diagnosis rather than an "Actual" diagnosis?

A. Because it contains three parts (PES).

B. Because it describes a problem that does not yet exist but has a high probability of occurring.

C. Because it is a medical diagnosis.

D. Because it focuses on the patient's strengths.

54. Which of the following data clusters leads to the most accurate nursing diagnosis for a patient with: Temp 39.5°C, flushed skin, and "I feel very hot"?

A. Risk for Infection related to high fever.

B. Hyperthermia related to infectious process as evidenced by increased body temperature.

C. Acute Pain related to warm skin.

D. Deficient Knowledge related to fever management.

55. After the Evaluation phase, if a nurse determines that a goal has been partially met but the patient still needs support, the most intelligent next step is to:

A. Terminate the plan of care.

B. Modify the plan of care to address unmet needs.

C. Start the initial assessment from the beginning.

D. Document that the patient failed the plan.

Essay Questions

Q1: List the two main types of data.

1. Subjective data (Symptoms).
2. Objective data (Signs).

Q2: List the three types of "Planning" in nursing.

1. Initial Planning.
2. Ongoing Planning.
3. Discharge Planning.

Q3: List the three types of "Nursing Interventions".

1. **Independent interventions:** Activities nurses initiate based on their own skills.
2. **Dependent interventions:** Activities carried out under a physician's orders.
3. **Collaborative interventions:** Actions performed in collaboration with other health team members.

Q4: List three purposes of the Nursing Process.

1. To identify a client's health status.
2. To identify actual or potential health care problems.
3. To establish plans to meet identified needs.
4. To deliver specific nursing interventions.



Q5: List main sources of data during assessment.

1. The Client (Primary source).
2. Support people (Family/Friends).
3. Client records (Medical history).
4. Health care professionals.

Q6: List three methods of data collection.

1. Observing.
2. Interviewing.
3. Examining (Physical assessment).

Q7: List the components of a NANDA Nursing Diagnosis.

1. Diagnostic Label (The problem).
2. Etiology (Related factors).
3. Defining Characteristics (Signs and symptoms).

Q8: List the two main focuses of the "Evaluation" phase.

1. The client's progress toward achievement of goals.
2. The effectiveness of the nursing care plan.

Q9: List the differences between "Nursing Diagnosis" and "Medical Diagnosis".

1. **Nursing Diagnosis:** Focuses on human responses, changes as the client's response changes.
2. **Medical Diagnosis:** Focuses on the disease process, remains the same as long as the disease is present.

Q10: List the five steps/phases of the Nursing Process in order:

1. **Assessment** (Collection of data).
2. **Diagnosis** (Identifying problems).
3. **Planning** (Setting goals, priorities and action).
4. **Implementation** (Taking action).
5. **Evaluation** (Reviewing results).

Q11: List the three possible outcomes/decisions after the "Evaluation" phase:

1. **Terminate the plan of care:** If the goals have been met.
2. **Modify the plan of care:** If the goals have not been met.
3. **Continue the plan of care:** If the goals are not yet met and more time is needed.

Q12: List characteristics of the Nursing Process:

1. **Cyclic and dynamic:** The steps are interrelated and continuous.
2. **Client-centered:** Focused on the unique needs of the individual.
3. **Interpersonal and collaborative:** Involves interaction with the client and health team.
4. **Focus on problem-solving:** Directed toward identifying and resolving problems.
5. **Use of critical thinking:** Requires clinical reasoning and judgment.

Q14: Scenario

During the Planning phase for a patient with multiple problems, you identify the following:

The patient is worried about who will feed their cat at home.

The patient has an oxygen saturation of 88% on room air.

The patient has a small red area on their heel.

Task: Using Maslow's Hierarchy of Needs, justify the order in which you will address these problems.

Answer:

Priority 1: Oxygen saturation of 88%. This is a Physiological Need (Airway/Breathing) and is life-threatening. It must be addressed first in the "Setting Priorities" step.

Priority 2: Red area on the heel. This relates to Safety and Physical Integrity (preventing a pressure ulcer).

Priority 3: Worry about the cat. This is a Psychological/Love and Belonging need. While important to the patient, it is the lowest priority according to the hierarchy of survival.

Q15: Scenario:

During an Initial Assessment, you find the following:

Patient's skin is hot and dry.

Patient's pulse is 115 bpm (Tachycardia).

Patient has not urinated in 10 hours.

The patient's wife says, "He hasn't been drinking much for two days."

Task: Create a complete 3-part Nursing Diagnosis (PES) based on this data and identify which piece of data is "Subjective" and which is "Secondary."

Answer:

Nursing Diagnosis: Deficient Fluid Volume (P) related to decreased oral intake (E) as evidenced by hot dry skin, tachycardia, and low urine output (S).

Subjective Data: "He hasn't been drinking much" (this is also Secondary Data because it came from the wife, not the patient).

Objective Data: Pulse 115 bpm, no urine for 10 hours, hot/dry skin (these are signs observed/measured by the nurse).

Q16: Scenario:

A patient is admitted to the medical ward with a diagnosis of pneumonia. Upon arrival, you perform a comprehensive head-to-toe examination to establish their baseline health status. Three hours later, you return specifically to check their lung sounds and oxygen levels because they complained of increased shortness of breath.

Task: Identify the two types of assessment performed in this scenario and justify your answer.

Answer:

Assessment 1: Initial Assessment.

Justification: This was performed after admission to establish a complete database for future comparison.

Assessment 2: Problem-focused Assessment.

Justification: This was done to determine the status of a specific respiratory problem identified earlier.



Q17: Scenario:

During a physical examination of a patient with a surgical wound, you gather the following information:

The patient states, "I feel very nauseated".

The patient's skin is pale and cool to the touch.

The patient's heart rate is 105 bpm.

The patient's sister mentions, "He didn't sleep at all last night because of the itching".

Task:

Classify each piece of data as Subjective or Objective, and identify which piece is Secondary Data.

Answer:

Subjective Data: "I feel very nauseated" (feelings/sensations described by the patient) and "He didn't sleep" (reported information).

Objective Data: Pale/cool skin (felt by the nurse) and Heart rate 105 bpm (measured by the nurse).

Secondary Data: The statement from the sister regarding his sleep and itching (information from a source other than the patient).

Q18: Scenario:

You are caring for a patient who had abdominal surgery yesterday.

Patient A: Has a large surgical incision, is crying, and rates their pain as 9/10.

Patient B: Has the same type of incision but is currently comfortable; however, you know that the incision could become infected if not cared for properly.

Task:

Create a 3-part Actual Nursing Diagnosis for Patient A and a 2-part Risk Nursing Diagnosis for Patient B.

Answer:

Patient A (Actual): Acute Pain (P) related to surgical wound (E) as evidenced by the patient crying and a pain scale of 9/10 (S).

Patient B (Risk): Risk for Infection (P) related to surgical incision (E). (Note: Risk diagnoses do not have "signs and symptoms" because the problem hasn't happened yet) .

Q19: Scenario:

To help a patient recover from surgery, the nurse performs the following:

Administers Morphine for pain as ordered by the surgeon.

Instructs the patient on how to perform deep breathing exercises.

Meets with the Dietician to plan a high-protein diet for the patient's wound healing.

Task:

Classify these interventions as Independent, Dependent, or Collaborative.

Answer:

Administering Morphine: Dependent Intervention (carried out under a physician's order).

Teaching breathing exercises: Independent Intervention (nurses are licensed to initiate this based on their knowledge).

Planning diet with Dietician: Collaborative Intervention (performed in coordination with other health team members).

Q20: Scenario: A nursing goal was set: "The patient will maintain an oxygen saturation (SpO2) of 95% or higher on room air within 24 hours". After 24 hours, the nurse checks and finds the patient's SpO2 is 90%.

Task:



Based on the Evaluation step of the nursing process, what are the three possible actions the nurse can take now?

Answer:

Modify the plan of care: Change the goal or interventions because the goal was not met.

Continue the plan of care: If the nurse believes more time is needed to reach the goal.

Terminate the plan of care: (Only if the goal had been met, which is not the case here).



Loss, Grief and death

Multiple Choice Questions

- An actual or potential situation in which something valued is changed or no longer available is called:
 - A. Grief.
 - ☒ B. Loss.
 - C. Bereavement.
 - D. Mourning.
- A loss that can be recognized by others is known as:
 - ☒ A. Actual loss.
 - B. Perceived loss.
 - C. Anticipatory loss.
 - D. Hidden loss.
- Which type of grief is experienced in advance of the event (e.g., before a loved one dies)?
 - A. Abbreviated grief.
 - ☒ B. Anticipatory grief.
 - C. Disenfranchised grief.
 - D. Unhealthy grief.
- The total response to the emotional experience related to loss is:
 - A. Death.
 - B. Denial.
 - ☒ C. Grief.
 - D. Anger.
- When a person cannot acknowledge their loss to others (e.g., loss of a socially unsanctioned relationship), it is called:
 - A. Chronic grief.
 - B. Inhibited grief.
 - ☒ C. Disenfranchised grief.
 - D. Exaggerated grief.
- According to Kübler-Ross, the first stage of dying is:
 - A. Anger.
 - ☒ B. Denial.
 - C. Bargaining.
 - D. Depression.



• In which stage of dying does the person try to make deals with God or a higher power to delay death?

- A. Acceptance.
- B. Anger.
- ☒ C. Bargaining.
- D. Depression.

• Which clinical sign indicates "Cessation of Circulation"?

- A. Dilated pupils.
- ☒ B. Absence of apical pulse.
- C. Loss of muscle tone.
- D. Drop in body temperature.

• The stiffening of the body that occurs 2 to 4 hours after death is called:

- A. Algor mortis.
- B. Livor mortis.
- ☒ C. Rigor mortis.
- D. Postmortem decomposition.

• While providing postmortem care, the nurse should place the body in which position?

- ☒ A. Supine with the head slightly elevated on a pillow.
- B. Prone to prevent swelling.
- C. Side-lying to drain secretions.
- D. Flat with no pillow.

• A patient feels they have lost their "independence" due to being in the hospital. This is an example of:

- A. Actual loss.
- ☒ B. Perceived loss. (فقدان يشعر به المريض ولا يراه الآخرون).
- C. Physical loss.
- D. Anticipatory loss.

Which postmortem change refers to the discoloration of the skin in dependent areas?

- A. Rigor mortis.
- B. Algor mortis.
- ☒ C. Livor mortis.
- D. Decomposition.

A patient who has undergone a mastectomy expresses that she feels she has lost her femininity. This is an example of:

A) External loss.

B) Loss of an aspect of oneself.

C) Environmental loss.

D) Perceived loss.



Which type of grief occurs when a person experiences sorrow in advance of an actual loss, such as a wife grieving for her terminally ill husband?

- A) Abbreviated grief.
- B) Disenfranchised grief.
- C) Anticipatory grief.**
- D) Pathologic grief.

During which stage of grieving might a client direct their frustration toward the nursing staff for minor issues?

- A) Denial.
- B) Bargaining.
- C) Anger.**
- D) Acceptance.

A dying patient says, "If I can just live long enough to see my daughter get married, I will be ready." This statement indicates which stage?

- A) Denial.
- B) Bargaining.**
- C) Depression.
- D) Acceptance.

What is the most appropriate nursing intervention for a client in the 'Depression' stage of grief?

- A) Encourage the client to talk and cheer them up.
- B) Use nonverbal communication, such as sitting quietly.**
- C) Avoid the client to give them privacy.
- D) Reinforce that the loss is happening.

Which of the following is a respiratory sign of impending clinical death?

- A) Rapid, deep, and regular breathing.
- B) Noisy breathing, known as the "death rattle."**
- C) Increased blood pressure.
- D) Warm and dry skin.

In which type of awareness do the client, family, and healthcare personnel know the prognosis is terminal but choose not to talk about it?

- A) Closed awareness.
- B) Mutual pretense.**
- C) Open awareness.
- D) Anticipatory awareness.

When performing postmortem care, why is a pillow placed under the head and shoulders of the deceased?

- A) To prevent rigor mortis.
- B) To prevent blood from discoloring the face.**
- C) To make the body easier to wrap in a shroud.
- D) To follow the hospital's legal policy.



The gradual decrease of the body's temperature after death is known as:

- A) Rigor mortis
- B) Algor mortis**
- C) Livor mortis
- D) Putrefaction

Which stage of grieving involves the client refusing to believe that the loss is happening?

- A) Anger
- B) Bargaining
- C) Denial**
- D) Acceptance

What is the primary difference between Hospice and Palliative care?

- A) Hospice is for any stage of illness, while Palliative is only for death.
- B) Hospice is for clients with a life expectancy of 6 months or less.**
- C) Palliative care does not involve pain management.
- D) Hospice care only focuses on the patient, not the family.

A nurse is caring for a patient who knows they are dying but chooses not to discuss it, while the family also knows but avoids the topic. This awareness pattern is:

- A) Closed awareness
- B) Mutual pretense**
- C) Open awareness
- D) Anticipatory grieving

Which nursing intervention is most appropriate for a client in the "Anger" stage who is shouting at the nurse?

- A) Explain to the client that their behavior is inappropriate.
- B) Withdraw from the room to allow the client to calm down.
- C) Allow the client as much control as possible over their care.**
- D) Defend the nursing staff against the client's complaints.

A client's family asks the nurse not to tell the client about their terminal diagnosis. Which action should the nurse take first?

- A) Respect the family's wishes and keep the diagnosis secret.
- B) Immediately tell the client their diagnosis to ensure "open awareness."
- C) Assess the family's cultural and religious beliefs regarding death.**
- D) Refer the matter to the hospital's legal department.

Which type of grief is "Abbreviated grief"?

- A) Long-term mourning for a spouse.
- B) Brief but genuine feelings when the lost object is easily replaced.**
- C) Grief that is hidden from society due to stigma.
- D) Grieving that occurs before the person actually dies.

When a nurse sits quietly with a grieving patient without expecting conversation, which stage is the patient likely in?

- A) Anger
- B) Bargaining
- C) Depression**
- D) Denial



A patient says: "I will go to therapy and stop smoking if I can just live to see my son's graduation." This indicates:

- A) Acceptance
- B) Bargaining**
- C) Denial
- D) Anger

A family from a specific ethnic group requests that the nurse does NOT tell the patient about their terminal prognosis. What should the nurse do?

- A) Tell the patient anyway because it is their legal right.
- B) Ignore the family and follow hospital policy only.
- C) Understand and respect the cultural belief to keep the patient free of worry.**
- D) Ask the doctor to force the family to tell the patient.

Which clinical sign indicates "Impending Clinical Death" related to the GI system?

- A) Increased appetite.
- B) Accumulation of flatus and abdominal distention.**
- C) Rapid digestion.
- D) Sharp, localized abdominal pain.

A patient says, "I will stop smoking and be a better person if I can just see my daughter's wedding." This reflects which stage?

- A) Denial
- B) Bargaining**
- C) Depression
- D) Acceptance

In which awareness pattern do the patient, family, and healthcare providers all know the prognosis is terminal and speak openly about it to finalize plans?

- A) Closed awareness
- B) Mutual pretense
- C) Open awareness**
- D) Anticipatory grief

Which of the following is an expected clinical manifestation of the "Loss of Muscle Tone" as death approaches?

- A) Increased ability to swallow.
- B) Relaxation of the facial muscles (jaw sagging).**
- C) Tightening of the abdominal muscles.
- D) Improved coordination of body movements.

When a client is in the "Acceptance" stage of grief, the nurse should expect the client to:

- A) Be very talkative and social.
- B) Act angry towards the staff.
- C) Be quiet and have decreased interest in the surrounding world.**
- D) Try to make deals with God for more time.



Scenario:

Mr. Ahmed is a 65-year-old patient with end-stage lung cancer. He frequently asks the nurse, "Why am I not getting better despite all these medications?" However, his family has strictly instructed the nursing staff NOT to tell him that his condition is terminal because they believe it will make him lose the will to live.

Q1: Based on the scenario above, What is the most significant nursing challenge or "burden" created by the awareness pattern described in the scenario?

- A) The patient will experience less pain.
- B) It facilitates open communication between the family and the nurse.
- C) It creates a barrier to effective communication and prevents the patient from finalizing personal affairs.**
- D) It simplifies the nursing care plan for the patient.

Q2 : How should the nurse respond to Mr. Ahmed's question: "Why am I not getting better?" while respecting both ethics and the family's current request?

- A) Tell him the truth immediately against the family's wishes.
- B) Lie and tell him that he will be better in a few days.
- C) Ask the patient what he understands about his illness to assess his readiness for the truth.**
- D) Ignore the question and leave the room.

Which of the following is an example of a "Perceived Loss"?

- A) Loss of a limb in an accident.
- B) Death of a family member.
- C) A student feeling a loss of confidence after a low grade.**
- D) Loss of a house due to a fire.

A nurse is caring for a client who is expected to die within days. The family is already crying and starting the mourning process. This is:

- A) Abbreviated grief
- B) Anticipatory grief**
- C) Disenfranchised grief
- D) Pathologic grief

A patient says, "The lab must have mixed up my samples; I can't have cancer." This represents:

- A) Anger
- B) Denial**
- C) Depression
- D) Bargaining

Which of the following is a sign of "Impending Clinical Death" related to the Circulatory System?

- A) Hypertension.
- B) Cold skin and cyanosis (bluish color) of extremities.**
- C) Strong, bounding peripheral pulses.
- D) Increased body temperature.



"Death Rattle" is a sound caused by:

- A) Rapid heart rate.
- B) Collection of mucus in the throat due to loss of gag reflex.**
- C) Stiffness of the chest muscles.
- D) Abdominal distention.

When is an autopsy (Postmortem examination) legally required?

- A) In every hospital death.
- B) Only if the nurse requests it.
- C) When the death is sudden or occurs within 24 hours of admission.**
- D) Only if the patient was over 80 years old.

In "Closed Awareness," who is kept unaware of the terminal prognosis?

- A) The family.
- B) The nursing staff.
- C) The patient.**
- D) The physician.

To evaluate if a dying patient feels "in control," the nurse should listen for reports of:

- A) High-tech medical interventions.
- B) Control over pain relief and treatment plans.**
- C) Total isolation from family.
- D) Strict adherence to hospital schedules.

A patient with chronic renal failure expresses sadness because he can no longer travel freely due to dialysis schedules. This is an example of:

- A) Actual loss of an external object.
- B) Loss of an aspect of oneself (physiologic function).**
- C) Environmental loss.
- D) Perceived loss.

Which of the following best describes "Perceived Loss"?

- A) A woman grieving after a house fire.
- B) A man mourning the death of his brother.
- C) A retired executive feeling a loss of prestige and social status.**
- D) A child losing a favorite toy.

A student nurse is caring for a patient whose wife died 3 years ago, yet he still cannot talk about her without intense, functional-impairing crying. This is:

- A) Abbreviated grief.
- B) Anticipatory grief.
- C) Complicated/Pathologic grief.**
- D) Disenfranchised grief.

During the "Anger" stage of Kübler-Ross, the nurse's most professional response to a shouting patient is:

- A) Explain that the staff is doing their best.
- B) Ask the family to calm the patient down.
- C) Recognize that the anger is a natural reaction to feeling powerless.**
- D) Set strict limits and leave the room.



A terminally ill patient begins to make funeral arrangements and says, "I want to make sure my kids are taken care of." This indicates:

- A) Depression.
- B) Bargaining.
- C) Acceptance.**
- D) Denial.

When an autopsy is required by law (e.g., sudden death), what is the nurse's priority regarding medical tubes (IVs, Catheters)?

- A) Remove all tubes immediately to clean the body.
- B) Cut all tubes to be flush with the skin.
- C) Leave all tubes in place according to agency policy for the examiner.**
- D) Ask the family if they want the tubes removed.

"Mutual Pretense" is often considered a communication barrier because:

- A) The patient does not know they are dying.
- B) The nurse is not informed of the diagnosis.
- C) It prevents honest discussion about the patient's fears and final wishes.**
- D) It is illegal in most hospital settings.

To evaluate if a dying patient's "Dignity" was maintained, the nurse looks for evidence of:

- A) The patient receiving the most expensive treatments.
- B) The patient's hair being groomed and the body being handled with respect after death.**
- C) The family being kept away from the patient during the final moments.
- D) The patient remaining unconscious during the dying process.

Essay Questions

Q1: List the four sources of loss.

1. Loss of an aspect of oneself (e.g., body part or function).
2. Loss of an object external to oneself.
3. Separation from an accustomed environment.
4. Loss of a loved or valued individual.

Q2: List three factors that influence the grief response.

1. Age and development.
2. Significance of the loss.
3. Culture and Spiritual beliefs.
4. Gender and Support systems.
5. Socioeconomic status.

Q3: List four clinical signs of death.

1. Total lack of response to external stimuli.
2. No muscular movement (especially breathing).
3. No reflexes (e.g., fixed and dilated pupils).
4. Flat encephalogram (brain death).



Q4: List the three postmortem changes that occur after death.

1. **Rigor mortis:** Stiffening of the body.
2. **Algor mortis:** Gradual decrease in body temperature.
3. **Livor mortis:** Discoloration of the skin in dependent areas.

Q5: List nursing responsibilities for postmortem care (Care of the body).

1. Place the body in a supine position with the head slightly elevated on a pillow.
2. Close the eyes and mouth.
3. Wash soiled areas of the body.
4. Place absorbent pads under the buttocks.
5. Remove all jewelry (except wedding bands which are taped).
6. Apply identification tags (wrist and outside the shroud).
7. Wrap the body in a shroud.

Q6: List two types of "Unhealthy Grief" (Complicated Grief).

1. Delayed grief (Inhibited).
2. Chronic grief (Extended in duration).
3. Exaggerated grief.

Q7: List two goals for a dying patient (Planning phase).

1. Maintaining physiological and psychological comfort.
2. Achieving a dignified and peaceful death.

Q8: List the four sources of loss.

1. Loss of an aspect of oneself (e.g., body part, physiologic function).
2. Loss of an object external to oneself (e.g., money, pets).
3. Separation from an accustomed environment (e.g., moving to a hospital).
4. Loss of a loved or valued individual (e.g., death of a spouse).

Q9: List the three types of loss.

1. **Actual loss:** Recognized by others.
2. **Perceived loss:** Experienced by the individual but cannot be verified by others.
3. **Anticipatory loss:** Experienced before the loss actually occurs.

Q10: List four types of "Grief".

1. **Abbreviated grief:** Brief but genuinely felt.
2. **Anticipatory grief:** Experienced in advance of the event.
3. **Disenfranchised grief:** When a person is unable to acknowledge the loss to others.
4. **Complicated/Unhealthy grief:** Strategies to cope with the loss are maladaptive.

Q11: List the five stages of dying according to Elisabeth Kübler-Ross.

1. **Denial:** Refusal to believe the loss is happening.
2. **Anger:** Directing anger toward staff or family.
3. **Bargaining:** Trying to make deals to delay death.
4. **Depression:** Profound sadness and withdrawal.
5. **Acceptance:** Coming to terms with the loss and feeling peace.



Q12: List four signs of "Approaching Death" (before the event).

1. Loss of muscle tone (relaxation of facial muscles, difficulty speaking).
2. Slowing of the circulation (cold skin, cyanosis, diminished sensation).
3. Changes in respirations (rapid, shallow, or irregular breathing).
4. Sensory impairment (blurred vision, impaired taste/smell).

Q13: List two nursing diagnoses for a dying client or their family.

1. Grieving related to the loss of a loved one.
2. Fear related to the unknown.
3. Hopelessness.
4. Caregiver Role Strain.

Q14: List two nursing goals for a dying patient (Planning).

1. Maintaining physiological and psychological comfort.
2. Achieving a dignified and peaceful death.

Q15: List the indicators for evaluating a client's "dignified death".

1. The client reports feeling in control of the environment (e.g., pain relief).
2. The client shares feelings about death.
3. The family members express their grief and support for each other.

Q16: List states of awareness in terminal illness

1. Closed Awareness: The client is not made aware of their impending death.
2. Mutual Pretense: Everyone (client, family, and staff) knows the prognosis is terminal but no one talks about it.
3. Open Awareness: Both the client and family know about the impending death and discuss it openly.



Stress & coping

Multiple Choice Questions:

A patient is experiencing prolonged stress. The nurse notes that the patient has developed deep ulcers in the stomach lining and enlarged lymph nodes. According to Selye's General Adaptation Syndrome (GAS), which stage is the patient most likely in?

- A) Alarm Reaction
- B) Stage of Resistance
- C) Stage of Exhaustion**
- D) Local Adaptation Syndrome

A nursing student is preparing for a final exam. To cope with the stress, the student starts regular physical exercise and practices time management to prioritize tasks. How should the nurse categorize these coping strategies?

- A) Short-term and Maladaptive
- B) Long-term and Adaptive**
- C) Emotion-focused and Maladaptive
- D) Stimulus-based and Situational

A 45-year-old client is admitted for a biopsy. While waiting for the procedure, the client's pupils dilate, heart rate increases, and the skin becomes pallid. The nurse recognizes these as physiologic indicators of stress. What is the primary cause of these specific manifestations?

- A) Release of mineral corticoids causing water retention.
- B) Decreased activity of the neuroendocrine system.
- C) Activation of the neuroendocrine systems for defense.**
- D) Development of the stage of resistance.

A client tells the nurse, "I've started drinking a few glasses of wine every night to help me forget about my work pressure." Which of the following is the most accurate nursing evaluation of this coping mechanism?

- A) It is a long-term strategy because it provides comfort.
- B) It is a short-term, maladaptive strategy that avoids reality.**
- C) It is a problem-focused strategy that improves the situation.
- D) It is an adaptive strategy that reduces stress to a tolerable limit.

During a physical examination, a nurse observes a client biting their nails and moving restlessly. The client states, "I just feel overwhelmed by my new job and moving to this city." The nurse correctly identifies these stressors as:

- A) Internal and Developmental
- B) External and Developmental
- C) External and Situational**
- D) Internal and Situational



A nurse is teaching a group of working parents about stress management. Which suggestion by the nurse demonstrates an effective Time Management strategy as a way to increase feelings of control?

- A) Avoiding all stressful situations at work.
- B) Increasing caffeine intake to stay alert.
- C) Delegating household tasks to other family members.**
- D) Using emotion-focused coping to feel better temporarily.

Which physiologic indicator of stress is a direct result of the body attempting to control elevated body heat during a stressful event?

- A) Increased sweat production (diaphoresis).**
- B) Constriction of peripheral blood vessels.
- C) Dilation of the bronchioles.
- D) Increased blood sugar levels.

What is an 'Intellectual' effect of stress?

- A. It causes negative feelings about the self.
- B. It challenges one's beliefs and values.
- C. It can influence problem-solving abilities.**
- D. It alters an individual's relationships with others.

Which of the following is a physical manifestation of stress that a nurse might observe during an examination?

- A. Increased spiritual faith.
- B. Nail biting and nervousness.**
- C. Enhanced time management skills.
- D. Accurate reality acceptance.

'Local Adaptation Syndrome' (LAS) is characterized by:

- A. The entire body reacting to a stressor simultaneously.
- B. One organ or a specific part of the body reacting alone.**
- C. The permanent exhaustion of all hormonal reserves.
- D. A purely psychological response to external pressure.

A nurse is advising a patient to avoid excesses of caffeine, salt, and sugar to minimize irritability and anxiety. This falls under which health promotion strategy?

- A. Exercise.
- B. Sleep management.
- C. Nutrition.**
- D. Time management.

Which of the following physiological changes occurs because of the dilation of the bronchioles during stress?

- A. Urinary frequency increases.
- B. Rate and depth of respirations increase.**
- C. Blood sugar levels decrease.
- D. Peripheral blood vessels dilate.



A patient feels "sad, despairing, and has a lack of worth" after losing their job. The nurse identifies this as:

- A. Anger.
- B. Depression.**
- C. Fear.
- D. Self-actualization.

When an individual uses 'discipline' to convey a sense of being in control during a crisis, it is helpful because it:

- A. Increases muscle tension for defense.
- B. Prevents panic and nonproductive actions.**
- C. Resolves the external stressor immediately.
- D. Eliminates the need for coping strategies.

Scenario: A nurse is teaching a client relaxation techniques to use when they encounter stressful health situations. Which techniques are useful?

- A. Increasing salts and sugars.
- B. Breathing exercises, massage, and yoga.**
- C. Alcohol consumption and drug use.
- D. Increasing caffeine and sugar intake.

A client who is "preoccupied with their own problems and unable to accept reality" is showing a negative effect of stress on which basic human need?

- A. Safety and security.
- B. Love and belonging.
- C. Self-actualization.**
- D. Physiologic needs.

Scenario: During the assessment, a nurse finds that a patient's mouth is dry and they are experiencing diarrhea. These symptoms are categorized as:

- A. Psychological indicators.
- B. Cognitive indicators.
- C. Physiologic signs and symptoms of stress.**
- D. Social effects of stress.

A nurse is helping a patient understand that stress can result from "positive" experiences like a new job or a birth of a child. These are examples of:

- A. Internal stressors.
- B. Situational stressors.**
- C. Biological stressors.
- D. Developmental stressors.

Which sequence correctly represents the steps of 'Problem-solving' as a cognitive indicator of stress?

- A. Evaluate -> Choose alternatives -> Carry out -> Assess.
- B. Assess -> Analyze/Define -> Choose alternatives -> Carry out -> Evaluate.**
- C. Analyze -> Evaluate -> Assess -> Choose alternatives.
- D. Choose alternatives -> Carry out -> Assess -> Analyze.



The release of mineral corticoids during stress leads to which physiological response?

- A. Increased urinary output.
- B. Sodium and water retention increase.**
- C. Decreased cardiac output.
- D. Relaxation of muscle tension.

Which of the following best describes a 'stressor'?

- A. The internal feeling of anxiety experienced by an individual.
- B. Any event or stimulus that causes an individual to experience stress.**
- C. The physiological homeostasis maintained by the body.
- D. A coping strategy used to minimize distress.

Which stage of the General Adaptation Syndrome (GAS) involves the body attempting to limit the stressor to the smallest area possible?

- A. Alarm Reaction
- B. Stage of Resistance**
- C. Stage of Exhaustion
- D. Stage of Recovery

What is the main goal of 'Models of Stress' in nursing practice?

- A. To eliminate all stressors from a patient's life.
- B. To assist nurses in predicting stressors and understanding individual responses.**
- C. To replace physical examinations with psychological testing.
- D. To diagnose medical conditions like hypertension.

Which physiological indicator is a result of the 'Alarm Reaction' phase?

- A. Decreased blood sugar.
- B. Constriction of pupils.
- C. Increased sweat production (diaphoresis).**
- D. Increased urinary output.

A student moves to a new city to start university. This is an example of which type of stressor?

- A. Internal stressor
- B. Biological stressor
- C. External situational stressor**
- D. Developmental stressor

A patient says, "I've started smoking more to deal with my financial problems." The nurse identifies this as:

- A. Adaptive coping
- B. Problem-focused coping
- C. Maladaptive short-term coping**
- D. Long-term constructive strategy



Which of the following is considered a 'Cognitive Indicator' of stress?

- A. Feelings of anger and fear.
- B. Activation of the neuroendocrine system.
- C. Problem-solving through specific steps to arrive at a solution.**
- D. Changes in appetite and sleep patterns.

How does 'Emotion-focused' coping differ from 'Problem-focused' coping?

- A. Emotion-focused changes the situation; problem-focused relieves feelings.
- B. Problem-focused coping does not improve the situation.
- C. Emotion-focused coping relieves emotional distress without changing the situation.**
- D. Both strategies are identical in their outcomes.

Scenario: A nurse is assessing a client scheduled for surgery. The client is pacing, has cold hands, and is breathing rapidly. Which nursing intervention is the priority based on 'Minimizing Anxiety'?

- A. Administering an immediate sedative.
- B. Encouraging the client to take deep breaths.**
- C. Leaving the client alone to allow them to regain self-control.
- D. Explaining the surgical procedure in technical detail.

Scenario: A working parent feels overwhelmed and is unable to manage household tasks. The nurse suggests delegating tasks to family members. This is an example of:

- A. Intellectual effect of stress.
- B. Time management as a coping strategy.**
- C. Spiritual effect of stress.
- D. Local Adaptation Syndrome (LAS).

When the body can no longer maintain the adaptation made during the resistance stage, and stress effects spread to the entire body, this is known as:

- A. The Alarm stage.
- B. The Local Adaptation Syndrome.
- C. The Stage of Exhaustion.**
- D. Homeostatic balance.

Scenario: A client reports that they have started exercising regularly and eating a healthy diet to manage job-related pressure. The nurse should document this as:

- A. Short-term maladaptive coping.
- B. Long-term adaptive coping strategies.**
- C. Stimulus-based stress response.
- D. Cognitive appraisal failure.

What is the effect of stress on an individual's pupils?

- A. Constriction to focus on details.
- B. Dilation to increase visual perception.**
- C. No change in pupillary size.
- D. Rapid blinking.



According to Selye, the "nonspecific response of the body to any kind of demand" is the definition of:

- A. Coping.
- B. Homeostasis.
- C. Stress.**
- D. Adaptation.

Which of the following is an example of an 'Internal' stressor?

- A. Death of a family member.
- B. Moving to a new apartment.
- C. Feelings of depression or an infection.**
- D. Pressure from peers at work.

Why does the heart rate and cardiac output increase during a stress response?

- A. To decrease the body's temperature.
- B. To transport nutrients and oxygen to the tissues.**
- C. To stimulate the digestive system.
- D. To promote better sleep patterns.

A patient is experiencing 'pallid' (pale) skin during an acute stress episode. This is caused by:

- A. Dilation of the bronchioles.
- B. Increased sweat production.
- C. Constriction of peripheral blood vessels.**
- D. Decreased blood sugar levels.

Which model of stress defines it as a life event or set of circumstances that increases vulnerability to illness?

- A. Response-based model.
- B. Stimulus-based model.**
- C. Transaction-based model.
- D. Local Adaptation Syndrome.

A nurse is helping a client prioritize tasks to decrease role demands. This intervention is part of:

- A. Nutrition management.
- B. Time management.**
- C. Physical exercise.
- D. Relaxation techniques.

Which nursing diagnosis is most appropriate for a family member who is struggling to provide constant care for a chronically ill relative?

- A. Cognitive impairment.
- B. Caregiver stress (Caregiver role strain).**
- C. Defensive denial.
- D. Altered elimination pattern.

Which of the following describes 'Maladaptive Coping'?

- A. Minimizing distress associated with stressful events.
- B. Dealing effectively with stressors.
- C. Causing unnecessary distress for the individual and others.**
- D. Using problem-solving to arrive at a solution.



When evaluating a client's response to stress management, what should the nurse use as a guide?

- A. The initial physical examination findings only.
- B. The desired outcomes developed during the planning stage.**
- C. The number of relaxation techniques the client learned.
- D. The presence of internal stressors only.

Which stage of the General Adaptation Syndrome (GAS) alerts the body's defenses?

- A. Stage of Resistance
- B. Stage of Exhaustion
- C. Alarm Reaction**
- D. Local Adaptation Syndrome

What happens to blood sugar levels during a stress response?

- A. Blood sugar decreases to conserve energy.
- B. Blood sugar stays the same.
- C. Blood sugar increases.**
- D. Blood sugar only changes during the exhaustion stage.

Which type of stressor occurs at predictable times throughout an individual's life?

- A. Situational stressors
- B. Developmental stressors**
- C. Internal stressors
- D. Environmental stressors

Which of the following is a "Social effect" of stress?

- A. Threatening physiologic homeostasis.
- B. Challenging one's beliefs and values.
- C. Producing negative feelings about the self.
- D. Altering an individual's relationships with others.**

Why is "Optimal Nutrition" essential during periods of high stress?

- A. It eliminates the need for physical exercise.
- B. It increases the body's resistance to stress.**
- C. It decreases the heart rate instantly.
- D. It prevents the Alarm Reaction from occurring.

A patient decides to start exercising regularly to relieve tension and feel better. This is an example of:

- A. A short-term maladaptive strategy.
- B. A long-term constructive coping strategy.**
- C. An intellectual indicator of stress.
- D. A stimulus-based model of stress.

When a person uses "Problem-focused coping," they are primarily trying to:

- A. Relieve emotional distress through thoughts.
- B. Ignore the situation until it goes away.
- C. Improve the situation by making changes or taking action.**
- D. Challenge their spiritual beliefs.



If a body's adaptation fails to overcome a stressor during the third stage of GAS, what is the possible ultimate consequence?

- A. The body returns to a state of resistance.
- B. The stressor is limited to a smaller area.
- C. Death may be the ultimate consequence.**
- D. The pupils will constrict.

A nurse encourages a client to take deep breaths before an injection. This is an example of:

- A. Assessing for nail biting.
- B. Minimizing anxiety through a relaxation technique.**
- C. Evaluating the success of a long-term strategy.
- D. Identifying a developmental stressor.

Stress is described as a "universal phenomenon" because:

- A. It only affects individuals in certain professions.
- B. All individuals experience it at some point.**
- C. It only results from negative experiences.
- D. It is found only in school environments.

What is the primary purpose of the "Alarm Reaction" stage?

- A. To rest and return the body to normal.
- B. To alert the body's defenses.**
- C. To exhaust all coping mechanisms.
- D. To limit stress to a single organ.

Which of the following is a "Physical effect" of stress?

- A. Challenging one's values.
- B. Threatening physiologic homeostasis.**
- C. Improving problem-solving abilities.
- D. Producing negative feelings about the self.

A patient feels overwhelmed and states, "I can't think clearly to solve my problems." This represents an effect of stress on which area?

- A. Social.
- B. Emotional.
- C. Intellectual.**
- D. Spiritual.

Which of the following is considered a "Long-term" adaptive coping strategy?

- A. Using alcoholic beverages to relax.
- B. Regularly exercising and eating a healthy diet.**
- C. Ignoring the situation until it passes.
- D. Using drugs to temporarily reduce stress.

During the "Stage of Resistance," the body's attempt to limit the stressor to a small area is an example of:

- A. General Adaptation Syndrome (GAS).
- B. Local Adaptation Syndrome (LAS).**
- C. Stimulus-based reaction.
- D. Exhaustion consequence.



Which nursing diagnosis is most relevant when a client's stress leads to "Isolation and withdrawal"?

A. Caregiver stress.

B. Anxiety.

C. Difficult coping related to social needs.

D. Knowledge deficit regarding nutrition.

When evaluating the success of stress management, the nurse's primary guide should be:

A. The number of medications administered.

B. The desired outcomes developed during the planning stage.

C. The presence of any external stressors.

D. The client's ability to avoid all stress.

Essay Questions

List the three main models of stress

1. Stimulus-based model.
2. Response-based model.
3. Transaction-based model.

List the three stages of the General Adaptation Syndrome (GAS) and the Local Adaptation Syndrome (LAS).

Answer:

1. Alarm reaction.
2. Resistance.
3. Exhaustion.

Enumerate five (5) physiologic indicators of stress.

1. Pupils dilate.
2. Sweat production (diaphoresis) increases.
3. Heart rate and cardiac output increase.
4. Skin becomes pallid.
5. Rate and depth of respirations increase.
6. Blood sugar increases.

List four (4) health promotion strategies that nurses can encourage to help clients manage stress.

1. Regular exercise.
2. Optimal nutrition.
3. Adequate rest and sleep.
4. Relaxation techniques
5. Time management.

Name the three (3) specific relaxation techniques that nurses can teach their patients.

1. Breathing exercises.
2. Massage.
3. Yoga.

List the types of stressors based on their origin

1. Internal stressors: Originate within the individual (e.g., infection or feelings of depression).
2. External stressors: Originate outside the individual (e.g., death in the family or pressure from peers).



List the types of stressors based on predictability

1. Developmental stressors: Occur at predictable times throughout life.
2. Situational stressors: Unpredictable and may occur at any time (e.g., new job or illness).

List the general categories of stressors:

1. Biological stressors (e.g., illness, disabilities).
2. Environmental stressors (e.g., poverty, overcrowding).

List the psychological indicators of stress:

1. Anxiety
2. Fear.
3. Anger (verbal expression of discomfort).
4. Depression (sadness, despair, or lack of worth).

List the cognitive indicators of stress:

1. Problem-solving (thinking through the situation).
2. Self-control or discipline (conveying a sense of being in control).

List negative effects on basic human needs

- Effects of stress on physiologic needs:
 1. Altered elimination patterns.
 2. Changes in appetite.
 3. Altered sleep patterns.
- Effects of stress on safety and security needs:
 1. Expressed nervousness.
 2. Feelings of being threatened.
- Effects of stress on love and belonging needs:
 1. Becoming isolated and withdrawn.
 2. Becoming overly dependent.
 3. Blaming others for own problems.
- Effects of stress on self-actualization needs:
 1. Preoccupation with own problems.
 2. Inability to accept reality.

List coping strategies based on their focus

1. Emotion-focused coping: Thoughts and actions that relieve emotional distress.
2. Problem-focused coping: Efforts to improve a situation by making changes or taking action to solve the problem or situation.

List coping strategies based on duration and effectiveness

1. Long-term coping strategies: Constructive and practical (e.g., healthy diet, regular exercise).
2. Short-term coping strategies: Reduce stress temporarily but are ineffective for reality (e.g., alcohol or drugs).

List coping strategies based on adaptation

1. Adaptive coping: Helps deal effectively with events and minimizes distress.
2. Maladaptive coping: Causes unnecessary distress for the individual or others.



List the different 'effects' of stress on an individual.

1. Physical effect.
2. Emotional effect.
3. Intellectual effect.
4. Social effect.
5. Spiritual effect.

List four (4) nursing diagnoses related to stress and coping.

1. Anxiety.
2. Caregiver stress.
3. Denial.
4. Difficult coping (individual or family).



Sleep

Multiple Choice Questions:

1. Which of the following hormones is inhibited during sleep and reaches its peak at the beginning of daylight?
A) Melatonin
B) Growth hormone
C) Cortisol
D) Thyroxine
2. Circadian regularity in humans typically begins to develop by the:
A) 1st week of life
B) 6th week of life
C) 1st year of life
D) 2nd year of life
3. What percentage of a night's sleep is typically spent in NREM sleep?
A) 25%
B) 50%
C) 75%
D) 90%
4. During which stage of sleep are the skeletal muscles most relaxed and snoring most likely to occur?
A) NREM Stage 1
B) NREM Stage 2
C) NREM Stage 3
D) REM Sleep
5. Which sleep disorder is characterized by more than five breathing pauses longer than 10 seconds per hour?
A) Narcolepsy
B) Hypersomnia
C) Insomnia
D) Sleep Apnea
6. A nurse is caring for a hospitalized client who is suddenly showing signs of agitation and confusion. Based on clinical alerts, this may be an initial presentation of:
A) Sleep apnea
B) Delirium due to sleep deprivation
C) Narcolepsy
D) Hypothyroidism
7. A 75-year-old patient reports waking up at 5:00 AM every day. Compared to a 25-year-old, this pattern is considered:
A) A symptom of chronic insomnia
B) A sign of clinical depression
C) A normal age-related change
D) A side effect of medication



8. Which of the following medications is specifically known to cause insomnia and nightmares by disrupting REM sleep?

- A) Antihistamines
- B) CNS stimulants
- C) Beta blockers**
- D) Antidepressants

9. When assessing a client for Obstructive Sleep Apnea (OSA), which physical finding is considered a predisposing factor in adults?

- A) Hypotension
- B) Obesity**
- C) Excessive weight loss
- D) Deviated ear canal

10. A client reports difficulty falling asleep for several nights, followed by a few nights of adequate sleep before the problem returns. The nurse documents this as:

- A) Acute insomnia
- B) Chronic-intermittent insomnia**
- C) Hypersomnia
- D) Parasomnia

11. Which nursing intervention is most appropriate to reduce environmental distractions for a client in a semi-private hospital room?

- A) Keep the overhead lights on for safety
- B) Increase the volume of the telephone ringtone
- C) Close the curtains between clients**
- D) Discourage the use of the client's own pajamas

12. When providing health teaching, the nurse should advise the client that exercise is most conducive to sleep when performed:

- A) Right before bedtime
- B) Late in the evening
- C) In the morning or early afternoon**
- D) Only during the night

13. Which of the following is a key question to ask when taking a sleep history?

- A) What is your job schedule?
- B) Do you have any bedtime rituals?**
- C) How much alcohol do you consume?
- D) Do you drive long distances?

14. In the first sleep cycle of an adult, NREM Stage 3 typically lasts for about:

- A) 5 to 10 minutes
- B) 20 to 30 minutes
- C) 50 to 60 minutes**
- D) 90 to 110 minutes



15. Which of the following best describes the physiological state during REM sleep?
- A) Heart and respiratory rates drop 20% to 30% below waking levels.
 - B) The brain is highly active, and metabolism may increase by 20%.**
 - C) The basal metabolic rate decreases by 10% to 30%.
 - D) Growth hormone levels reach their peak.
16. A sleeper who can be readily awakened and might deny they were even sleeping is most likely in:
- A) NREM Stage 1.**
 - B) NREM Stage 2.
 - C) NREM Stage 3.
 - D) REM Sleep.
17. What is the correct sequence of sleep cycles in a healthy adult?
- A) Cycles typically last 60 minutes and occur only in NREM.
 - B) Individuals pass through NREM and REM, with cycles lasting 90 to 110 minutes.**
 - C) Sleepers start with REM and then move to NREM Stage 3.
 - D) Cycles are irregular and do not follow a specific time frame in adults.
18. By which age do most infants establish a pattern of daytime naps and sleep through the night (midnight to 5 a.m.)?
- A) 6 weeks.
 - B) 3 months.
 - C) 6 months.**
 - D) 1 year.
19. Which type of sleep apnea involves a defect in the respiratory center of the brain where all breathing actions cease?
- A) Obstructive apnea.
 - B) Central apnea.**
 - C) Mixed apnea.
 - D) Hypersomnia.
20. A nurse is planning care for a client with narcolepsy. Which medication is commonly used to reduce excessive daytime sleepiness?
- A) Beta blockers.
 - B) Methylphenidate (Ritalin).**
 - C) Antihistamines.
 - D) Narcotics.
21. When assessing a client for "Insufficient Sleep," which symptoms should the nurse look for?
- A) Increased motivation and high energy.
 - B) Attention deficits, distractibility, and malaise.**
 - C) Excessive weight gain and snoring.
 - D) Improved performance on newly learned tasks.
22. Which of the following is a "Stimulus Control" therapy intervention for insomnia?
- A) Limiting time in bed to increase sleep drive.
 - B) Creating a sleep environment that promotes sleep.**
 - C) Learning to develop positive thoughts about sleep.
 - D) Using sedatives on an as-needed basis.



23. During sleep, which hormone is secreted to support tissue repair and growth?

- A) Cortisol
- B) Melatonin
- C) Growth hormone**
- D) Thyroid stimulating hormone

24. At what age do most infants begin to establish a regular sleep-wake cycle (circadian regularity)?

- A) 1st week of life
- B) 3 to 6 months**
- C) 12 months
- D) 2 years

25. Which stage of sleep is primarily responsible for restoring the mind and increasing brain metabolism?

- A) NREM Stage 1
- B) NREM Stage 2
- C) NREM Stage 3
- D) REM Sleep**

26. In a healthy adult, a complete sleep cycle (NREM + REM) typically lasts about:

- A) 30 to 60 minutes
- B) 90 to 110 minutes**
- C) 4 to 6 hours
- D) 20 minutes

27. A patient who is easily awakened and may deny they were even sleeping is likely in which stage?

- A) NREM Stage 1**
- B) NREM Stage 2
- C) NREM Stage 3
- D) REM sleep

28. What is the most common physiological change observed during NREM sleep?

- A) Increased pulse rate
- B) Peripheral blood vessel constriction
- C) Decrease in blood pressure and cardiac output**
- D) Increase in basal metabolic rate

29. Which condition is defined as excessive daytime sleepiness despite getting enough sleep at night?

- A) Insomnia
- B) Hypersomnia**
- C) Sleep apnea
- D) Parasomnia

30. "Sleep attacks" occurring between the ages of 15 and 30 are a hallmark manifestation of:

- A) Obstructive sleep apnea
- B) Narcolepsy**
- C) Acute insomnia
- D) Chronic fatigue syndrome



31. A patient with obstructive sleep apnea often wakes up because of:

- A) High melatonin levels
- B) Increased carbon dioxide levels in the blood**
- C) Low cortisol levels
- D) Dreaming during REM sleep

32. Which factor is considered one of the greatest causes of difficulty in falling or staying asleep?

- A) Diet
- B) Emotional stress**
- C) Weight loss
- D) Moderate morning exercise

33. What is the recommended sleep duration for a toddler (1 to 3 years old)?

- A) 16 to 18 hours
- B) 12 to 14 hours**
- C) 8 to 10 hours
- D) 7 to 9 hours

34. Which nursing intervention helps reduce environmental distractions in a hospital setting?

- A) Keeping communal televisions on all night
- B) Using high-volume telephone ringtones
- C) Warming the bed with prewarmed blankets
- D) Closing window curtains to block streetlights**

35. Which medication class is used to reduce excessive daytime sleepiness in narcolepsy patients?

- A) Beta blockers
- B) CNS stimulants (e.g., Ritalin)**
- C) Antidepressants
- D) Narcotics

35. When assessing a client's sleep history, which question is most important to ask regarding their night?

- A) "What did you eat for lunch?"
- B) "Do you have any problems with your sleep?"**
- C) "Do you like the hospital food?"
- D) "How many siblings do you have?"

36. Signs that an individual is not getting enough sleep include:

- A) Excellent concentration
- B) Unreasonable irritability and short-term memory loss**
- C) High motivation levels
- D) Feeling alert and refreshed upon awakening

37. Which hormone is at its highest level in the body when daylight begins to stimulate the individual?

- A) Melatonin
- B) Cortisol**
- C) Growth Hormone
- D) Thyroid Stimulating Hormone (TSH)



38. According to the clinical alert, sleep deprivation in hospitalized clients is a significant contributor to:

- A) Hypertension
- B) Delirium (agitation or confusion)**
- C) Diabetes type 2
- D) Weight gain

39. Sleep deprivation results in a significant increase in which hormone, leading to tension and hostility?

- A) Cortisol
- B) Growth Hormone
- C) Thyroid Stimulating Hormone (TSH)**
- D) Melatonin

40. In which stage of NREM sleep does the body temperature fall and the sleeper requires more intense stimuli (like touching) to awaken?

- A) Stage 1
- B) Stage 2**
- C) Stage 3
- D) Stage 4

41. During NREM Stage 3, the sleeper's heart and respiratory rates drop by:

- A) 5% to 10%
- B) 10% to 15%
- C) 20% to 30%**
- D) 40% to 50%

42. Which stage of sleep is considered essential for restoring physical energy and releasing growth hormones?

- A) REM sleep
- B) NREM Stage 1
- C) NREM Stage 2
- D) NREM Stage 3**

43. A preschooler (3 to 5 years of age) generally requires how many hours of sleep per night?

- A) 14 to 16 hours
- B) 11 to 13 hours**
- C) 9 to 10 hours
- D) 7 to 8 hours

44. Which age group is most likely to experience lower grades and increased potential for car crashes due to sleepiness?

- A) Toddlers
- B) School-age children
- C) Adolescents**
- D) Older adults

45. Older adults (65 to 75 years) tend to awaken an average of how many times during the night?

- | A) Once | B) Three times | **C) Six times** | D) Ten times |



46. Narcolepsy is primarily caused by a lack of which chemical in the Central Nervous System?

- A) Dopamine
- B) Serotonin
- C) Hypocretin**
- D) Acetylcholine

47. Which type of sleep apnea is associated with obesity, enlarged tonsils, or a deviated nasal septum?

- A) Central apnea
- B) Obstructive apnea**
- C) Mixed apnea
- D) Hypersomnia

48. In Mixed Apnea, the condition is a combination of:

- A) Insomnia and Narcolepsy
- B) Obstructive and Central apnea**
- C) Hypersomnia and Sleep deprivation
- D) REM and NREM disorders

49. Sleep quality is a subjective characteristic often determined by:

- A) Total hours slept
- B) Whether the individual wakes up feeling energetic**
- C) The number of REM cycles
- D) The absence of snoring

50. Weight gain is typically associated with:

- A) Increased total sleep time
- B) Reduced total sleep time and earlier awakening**
- C) Better sleep quality
- D) Longer REM periods

51. Nicotine has what effect on the body regarding sleep?

- A) Sedative effect
- B) Stimulating effect**
- C) No effect
- D) Induces deep Stage 3 sleep

52. Which of the following is a recommended intervention for enhancing sleep in a hospital?

- A) Keeping the room bright
- B) Encouraging the client to wear their own clothing (pajamas)**
- C) Waking the client frequently for routine checks
- D) Keeping the window curtains open

53. "Sleep restriction" as a treatment for insomnia involves:

- A) Avoiding all sleep for 24 hours
- B) Limiting time in bed to increase the need for sleep**
- C) Taking high doses of antihistamines
- D) Sleeping only during the day



54. A "Sleep Diary" should typically include information about:

- A) Daily calorie intake
- B) Consumption of caffeinated beverages and alcohol**
- C) Weekly exercise routines only
- D) Financial worries

55. When evaluating sleep medication safety, antihistamines should NOT be recommended for clients with:

- A) Back pain
- B) Cardiovascular disease or hypertension**
- C) Hypothyroidism
- D) Obesity

56. When assessing a client's sleep quality, the nurse understands that "Sleep Quality" is best defined as:

- A) Sleeping exactly 8 hours without waking up.
- B) A subjective characteristic determined by whether the client feels energetic upon awakening.**
- C) The total time spent in REM sleep specifically.
- D) The absence of any snoring or movement during the night.

57. A nurse is reviewing a client's "Sleep Diary." Which of the following entries is most helpful for diagnosing sleep disturbances?

- A) Daily intake of calories and types of food.
- B) Times of using medications, alcohol, or caffeinated beverages.**
- C) The number of steps walked during the day.
- D) The client's favorite bedside reading material.

58. During a physical examination for sleep concerns, the nurse should specifically assess for:

- A) Skin turgor and capillary refill.
- B) Deviated nasal septum, enlarged tonsils, or obesity.**
- C) Bowel sounds and abdominal tenderness.
- D) Deep tendon reflexes and cranial nerve function.

59. Which of the following is a primary "Nursing Diagnosis" for a client with persistent difficulty falling asleep?

- A) Chronic confusion.
- B) Insomnia related to personal stressors.**
- C) Risk for infection.
- D) Impaired physical mobility.

60. The major goal for a client with "Impaired Sleep Pattern" is to:

- A) Increase the dose of sedative medications.
- B) Sleep only during daytime hours to avoid noise.
- C) Maintain a sleep-wake pattern that provides sufficient energy for daily activities.**
- D) Spend at least 12 hours in bed regardless of sleep quality.

61. To reduce environmental distractions in a hospital, the nurse should:

- A) Keep the room temperature very high.
- B) Lower the ringtone of nearby telephones.**
- C) Keep all overhead lights on for safety.
- D) Encourage the use of hospital gowns instead of personal pajamas.



62. Which intervention promotes a "Sleep-Inducing Environment" before bed?

- A) Providing a large, heavy meal right before sleep.
- B) Warming the bed with prewarmed bath blankets.**
- C) Encouraging vigorous exercise an hour before bedtime.
- D) Keeping the window curtains open to see streetlights.

63. Sedative-hypnotic medications are primarily prescribed to:

- A) Increase blood pressure.
- B) Induce sleep.**
- C) Treat respiratory infections.
- D) Increase daytime alertness.

64. Why should antihistamines be avoided in clients with cardiovascular disease or hypertension?

- A) They cause excessive weight gain.
- B) They can lead to side effects like dizziness and further blood pressure complications.**
- C) They induce too much deep sleep (Stage 3).
- D) They increase the risk of sleep apnea.

65. When evaluating the effectiveness of a sleep intervention, the most reliable indicator of success is:

- A) The client's report of feeling refreshed and alert during the day.**
- B) The nurse's observation of the client lying still in bed.
- C) The fact that the client did not ask for pain medication.
- D) A decrease in the client's heart rate during the night.

66. Which of the following is a "Stimulus Control" instruction for a patient with insomnia?

- A) Read or watch TV in bed to get tired.
- B) Use the bed only for sleep and sexual activity.**
- C) Take long naps during the afternoon to make up for lost sleep.
- D) Drink a cup of strong coffee before trying to sleep.

67. Cognitive therapy for sleep focuses on:

- A) Restricting the amount of time spent in bed.
- B) Learning to develop positive thoughts and beliefs about sleep.**
- C) Using herbal supplements instead of drugs.
- D) Monitoring brain waves during REM sleep.

Essay Questions:

List reasons for sleep

- To prevent fatigue
- To conserve energy
- To restore the mind and body
- To enjoy life more fully.
- To enhances daytime functioning
- To vital for cognitive, physiologic, and psychosocial function.
- Allows an individual to awaken feeling alert and refreshed
- Important factor in an individual's quality of life
- To cope with daily stresses



List physiologic changes during non-rapid eye movement (NREM) sleep

- Blood pressure falls.
- Pulse rate decreases.
- Peripheral blood vessels dilate.
- Cardiac output decreases.
- Skeletal muscles relax.
- Basal metabolic rate decreases 10% to 30%.
- Growth hormone levels peak.
- Intracranial pressure decreases.

List signs that may indicate that an individual is not getting enough sleep

- Falling asleep or becoming drowsy during a task
- Not being able to concentrate or remember information
- Being unreasonably irritable with others.
- Contributes to short-term memory loss
- Inadequate performance on newly learned tasks.

What is Sleep Apnea and what are its three types?

- Characterized by breathing pauses longer than 10 seconds (more than 5 times per hour).
 1. Obstructive: Airway blocked (e.g., enlarged tonsils, obesity).
 2. Central: Defect in the brain's respiratory center.
 3. Mixed: Combination of both.

List the three types of Insomnia based on duration and pattern.

1. Acute insomnia.
2. Chronic insomnia.
3. Chronic-intermittent insomnia.

List four common symptoms of Insomnia.

1. Difficulty falling asleep.
2. Waking up too early or frequently.
3. Difficulty concentrating.
4. Irritability and fatigue.

List three non-pharmacological treatments (Behavior patterns) for insomnia.

1. Stimulus control.
2. Cognitive therapy.
3. Sleep restriction.

List nursing interventions to promote sleep in a hospital setting

1. Close window curtains to block streetlights.
2. Close curtains between clients in semiprivate rooms.
3. Lower the ringtone of nearby telephones.
4. Reduce or eliminate overhead lighting.
5. Provide comfort measures (extra blankets, warm the bed).
6. Support bedtime rituals (reading, praying, hot drinks).



List the factors that the nurse should assess during a physical examination for sleep potential problems.

1. Enlarged tonsils.
2. Deviated nasal septum.
3. Obesity (thickened neck).
4. (In children) size of adenoids.

List four risk factors for Insomnia.

1. Older age.
2. Female gender (hormonal changes).
3. Medical conditions.
4. Personal stressors or worry.

List three types of medications or substances that interfere with sleep.

1. Beta blockers (cause nightmares/insomnia).
2. Narcotics (suppress REM sleep).
3. Stimulants like Caffeine or Nicotine.

Describe the three stages of NREM sleep:

Stage 1: Very light sleep, lasts a few minutes, person may deny they were asleep.

Stage 2: Body processes slow down, requires more intense stimuli (like touching) to awaken.

Stage 3: Deepest sleep, heart and respiratory rates drop 20-30%, essential for energy restoration and growth hormone



Fluids & electrolytes

1. Homeostasis refers to:

- A. Disease state
 - B. Balance of internal environment ✓**
 - C. Infection
 - D. Metabolism
-

2. Total body water in adults is approximately:

- A. 30%
 - B. 40%
 - C. 60% ✓**
 - D. 80%
-

3. Intracellular fluid (ICF) represents:

- A. 1/3
 - B. 2/3 ✓**
 - C. 1/4
 - D. 3/4
-

4. Extracellular fluid (ECF) represents:

- A. 1/3 ✓**
 - B. 2/3
 - C. 1/2
 - D. 3/4
-

5. Interstitial fluid is:

- A. Inside cells
 - B. In blood
 - C. Between cells ✓**
 - D. In bone
-



6. Plasma is part of:

A. ICF

B. ECF (intravascular) ✓

C. TCF

D. Lymph

7. Transcellular fluid includes:

A. Blood

B. CSF ✓

C. Muscle

D. Fat

8. The major intracellular cation is:

A. Sodium

B. Potassium ✓

C. Calcium

D. Chloride

9. The major extracellular cation is:

A. Potassium

B. Sodium ✓

C. Magnesium

D. Phosphate

10. Osmosis is movement of:

A. Solute

B. Water ✓

C. Protein

D. Cells

11. Diffusion occurs from:

A. Low to high

B. High to low ✓

C. Equal

D. Random



12. Filtration depends on:

- A. Osmotic pressure
 - B. Hydrostatic pressure ✓**
 - C. Temperature
 - D. pH
-

13. Active transport requires:

- A. Oxygen
 - B. ATP ✓**
 - C. Water
 - D. Enzymes
-

14. Sodium-potassium pump moves:

- A. Na inside
 - B. K outside
 - C. Na out & K in ✓**
 - D. Both in
-

15. Normal urine output is:

- A. 0.1 ml/kg/hr.
- B. 0.5–2 ml/kg/hr. ✓**
- C. 5 ml/kg/hr.
- D. 10 ml/kg/hr.

16. Daily fluid requirement is:

- A. 10 ml/kg
 - B. 20 ml/kg
 - C. 30–40 ml/kg ✓**
 - D. 60 ml/kg
-

17. First spacing means:

- A. Edema
 - B. Normal distribution ✓**
 - C. Ascites
 - D. Dehydration
-



18. Second spacing refers to:

A. Plasma

B. Edema ✓

C. Cells

D. Bone

19. Third spacing example:

A. Blood

B. Ascites ✓

C. Urine

D. Sweat

20. Normal pH is:

A. 6.5–7

B. 7.35–7.45 ✓

C. 7.5–8

D. 8–9

21. Acidosis occurs when pH is:

A. >7.45

B. <7.35 ✓

C. =7.4

D. >8

22. Alkalosis occurs when pH is:

A. <7

B. <7.35

C. >7.45 ✓

D. =7.4

23. Normal PaCO₂ is:

A. 20–30

B. 35–45 mmHg ✓

C. 50–60

D. 70–80



24. Increased PaCO_2 causes:

- A. Respiratory alkalosis
 - B. Respiratory acidosis** ✓
 - C. Metabolic alkalosis
 - D. Normal
-

25. Normal HCO_3^- is:

- A. 10–15
- B. 18–20
- C. 22–26 mEq/L** ✓
- D. 30–40

26. Increased HCO_3^- causes:

- A. Acidosis
 - B. Metabolic alkalosis** ✓
 - C. Respiratory acidosis
 - D. Shock
-

27. Kidneys regulate:

- A. Oxygen
 - B. Fluid & electrolytes** ✓
 - C. Temperature
 - D. Hormones
-

28. Lungs remove about:

- A. 100 ml water
 - B. 300 ml water/day** ✓
 - C. 1 liter
 - D. 2 liters
-

29. ADH function:

- A. Increase urine
 - B. Water retention** ✓
 - C. Sodium loss
 - D. Potassium increase
-



30. Aldosterone causes:

- A. Sodium loss
- B. Sodium retention** ☒
- C. Potassium retention
- D. Water loss

31. Normal BUN:

- A. 5–10
 - B. 10–20 mg/dl** ☒
 - C. 20–30
 - D. 30–40
-

32. Capillary refill normal time:

- A. <2 sec** ☒
 - B. 5 sec
 - C. 10 sec
 - D. 20 sec
-

33. Skin turgor assesses:

- A. Nutrition
 - B. Hydration** ☒
 - C. Pain
 - D. Oxygen
-

34. Urine color normally:

- A. Dark
 - B. Pale straw** ☒
 - C. Red
 - D. Blue
-

35. Hypokalemia means:

- A. High K
- B. Low K** ☒
- C. High Na
- D. Low Na



36. Hyponatremia means:

A. Low Na

B. High Na ✓

C. Low K

D. High Ca

37. Calcium normal function:

A. Digestion

B. Bone formation ✓

C. Oxygen transport

D. Immunity

38. Diffusion rate increases with:

A. Low temperature

B. High temperature ✓

C. Large molecules

D. Low gradient

39. Best indicator of fluid status:

A. BP

B. Weight change ✓

C. Pulse

D. Temperature

40. Osmotic diuresis is caused by:

A. Water intake

B. Glucose in urine ✓

C. Sodium loss

D. Protein intake

41. Infants have higher body water because of:

A. Low metabolism

B. High fat

C. High metabolic rate ✓

D. Low kidney function



42. Obese individuals have:

- A. More water
 - B. Less water percentage ✓**
 - C. Equal water
 - D. No change
-

43. Females generally have:

- A. More body water
 - B. Less body water than males ✓**
 - C. Same
 - D. Double
-

44. Which fluid is the smallest ECF component?

- A. Plasma
 - B. Interstitial
 - C. Transcellular ✓**
 - D. Lymph
-

45. Which hormone is released from posterior pituitary?

- A. Aldosterone
- B. ADH ✓**
- C. Cortisol
- D. Thyroxine

46. Parathyroid hormone regulates:

- A. Sodium
 - B. Calcium ✓**
 - C. Potassium
 - D. Chloride
-

47. Diffusion example includes:

- A. Blood filtration
 - B. Oxygen exchange in lungs ✓**
 - C. Urine formation
 - D. Sodium pump
-



48. A patient has ABG results: pH = 7.30, PaCO₂ = 50 mmHg, HCO₃⁻ = 24 mEq/L

What is the condition?

A. Metabolic acidosis

B. Respiratory acidosis ✓

C. Respiratory alkalosis

D. Metabolic alkalosis

49. A patient has ABG results: pH = 7.50, PaCO₂ = 30 mmHg, HCO₃⁻ = 24.

What is the condition?

A. Respiratory alkalosis ✓

B. Metabolic alkalosis

C. Respiratory acidosis

D. Normal

50. A patient has ABG results: pH = 7.25, PaCO₂ = 40 mmHg, HCO₃⁻ = 18

What is the condition?

A. Metabolic acidosis ✓

B. Respiratory acidosis

C. Respiratory alkalosis

D. Normal

51. A patient has ABG results: pH = 7.48, PaCO₂ = 40 mmHg, HCO₃⁻ = 30

What is the condition?

A. Metabolic alkalosis ✓

B. Respiratory alkalosis

C. Respiratory acidosis

D. Normal

52. A patient has ABG results: pH = 7.55, PaCO₂ = 45 mmHg, HCO₃⁻ = 32

What is the condition?

A. Respiratory alkalosis

B. Metabolic alkalosis ✓

C. Respiratory acidosis

D. Normal



53. A patient has ABG results: $\text{pH} = 7.40$, $\text{PaCO}_2 = 35 \text{ mmHg}$, $\text{HCO}_3^- = 22$

What is the condition?

- A. Fully compensated respiratory alkalosis
 - B. Normal ABG ✓**
 - C. Metabolic acidosis
 - D. Respiratory acidosis
-

54. List functions of body fluids.

- Carry nutrients and waste.
 - Facilitating digestion and promoting elimination.
 - Act as a lubricant in joints.
 - Give shape and form to the cells.
 - Regulate body temperature.
-

55. List body fluid compartments.

The total body water is distributed primarily between 2 compartments, namely, extracellular fluid (ECF) and intracellular fluid (ICF) compartments.

ECF is present outside cells:

- Makes up about $\frac{1}{3}$ of total body water, 20% of total body weight and includes:
- Intravascular fluid ($\frac{1}{4}$ of ECF): primary component of plasma
- Interstitial fluid ($\frac{3}{4}$ of ECF): lies outside blood vessels.
- Trans cellular (TCF)

ICF is present inside cells:

- Makes up about $\frac{2}{3}$ of total body water and 40% of body weight.
-

56. List factors affecting body fluid.

- ✓ **Age:** An infant especially on born prematurely has considerably more body fluid than an elderly person due to greater metabolic rate.
- ✓ **Lean body mass:** Fat tissues contain a small amount of water whereas lean tissue is rich in water, the more obese the person, the smaller the percentage of total body water.
- ✓ **Sex or gender:** Females tend to have proportionally more body fat than males; they also have less body fluid than males.
- ✓ **Environment:** Excess heat stimulates the sympathetic nervous system and cause person to sweat.
- ✓ **Diet:** In nutritional deficiency, the body preserved the protein, by breaking down the fat and glycogen.



- ✓ **Stress:** Water retention and increase the production of antidiuretic hormone.
 - ✓ **Illness:** As burn by fluid and electrolytes loss, and as renal disorders.
-

57. List types of fluid spacing.

First Spacing: describes normal distribution of fluid in the body in both the intracellular and extracellular fluid compartments

Second Spacing: describes the excess accumulation of fluid in the interstitial spaces, which we also call edema.

Third Spacing: occurs when fluid accumulates in areas that normally have no fluid or minimal amount of fluid, such as with ascites, and edema associated with burns. In extreme cases third spacing can cause a relative hypovolemia.

58. List mechanisms of fluid movement.

osmosis, diffusion, filtration, active transport

59. List major extracellular electrolytes.

Na, Cl, Ca, HCO₃

60. List major intracellular electrolytes.

K, Mg, phosphate

61. List organs regulating fluid balance.

kidneys, lungs, heart, pituitary, adrenal, parathyroid

62. List signs of dehydration.

thirst, poor skin turgor, low urine, weight loss

63. List types of ECF.

- **The intravascular space (IVF):** plasma; ~1/4 ECF: 3 L of the average 6 L of blood volume is made up of plasma, the remaining 3 L is made up of erythrocytes, leukocytes, and thrombocytes.
 - **Interstitial fluid (ISF): between cells;** ~3/4 ECF. The interstitial space contains the fluid that surrounds the cell. Lymph is an interstitial fluid.
 - **Trans cellular (TCF):** is the smallest division of the ECF compartment and contains approximately 1 L of ECF. Examples of trans cellular fluids include cerebrospinal, pericardial, synovial, intraocular, and pleural fluids; sweat; peritoneal and digestive secretions.
-

64. List functions of ADH.

water retention, regulate osmotic pressure, control blood volume



65. List effects of aldosterone.

sodium retention, water retention, potassium loss

66. List methods of assessing fluid balance.

- Clinical assessment
 - Review of fluid balance charts
 - Review of blood chemistry
-

67. List signs of fluid overload.

edema, weight gain, hypertension



Questions about nutrition lecture

Multiple choice questions:

1. Macronutrients means:
 - A. Nutrients that the body requires in large amounts
 - B. Nutrients that the body requires in small amounts
 - C. Nutrients not required by the body
 - D. Nutrients that the body requires in medium amounts

2. Which nutrient from below considered as micronutrients:
 - A. Carbohydrates
 - B. Fats
 - C. Proteins
 - D. Vitamins

3. The main source of energy is.....
 - A. Carbohydrates
 - B. Fats
 - C. Proteins
 - D. Minerals

4. Types of amino acids must be obtained from food:
 - A. Non-essential
 - B. Essential
 - C. Fat soluble
 - D. Water soluble

5. Type of fat considered as healthier fats
 - A. Saturated
 - B. Unsaturated
 - C. Trans
 - D. Complex

6. Fat soluble vitamins include:
 - A. Vitamins A, E, D, k
 - B. Vitamin B
 - C. Vitamin C
 - D. Calcium

7. Vitamins they need to be replenished regularly:
 - A. Fat soluble
 - B. Water soluble
 - C. Trans
 - D. Saturated

8. Fat soluble vitamin essential for vision and skin health:
 - A. Vitamin A
 - B. Vitamin D
 - C. Vitamin E
 - D. Vitamin K



9. Fat soluble vitamin essential for blood clotting:

- A. Vitamin A
 - B. Vitamin D
 - C. Vitamin E
 - D. Vitamin K
-

10. Calcium is considered as:

- A. Major mineral
 - B. Minor mineral
 - C. Macronutrients
 - D. Water soluble
-

11. Iron is considered as:

- A. Major mineral
 - B. Minor mineral
 - C. Macronutrients
 - D. Water soluble
-

12. Enteral nutrition refers to:

- A. Oral nutrition only
 - B. Tube feeding
 - C. IV nutrition
 - D. Vitamin supplements
-

13. One important role of nurses in nutrition is:

- A. Prescribing medications
 - B. Providing patient education
 - C. Performing surgery
 - D. Manufacturing food
-

14. BMI above 30 indicates:

- A. Underweight
 - B. Normal weight
 - C. Overweight
 - D. Obesity
-

15. BMI is calculated using:

- A) Weight / height
 - B) Weight (kg)/ height (m²)
 - C) Height / weight
 - D) Waist / hip
-

16. Which of the following is an anthropometric measurement?

- A. Hemoglobin test
 - B. BMI
 - C. Vitamin D test
 - D. Albumin test
-

17. Proteins are made up of:

- A) Fatty acids
 - B) Amino acids
 - C) Minerals
 - D) Glucose
-



18. Simple carbohydrates are characterized by:

- A) Slow digestion
 - B) Rapid digestion and quick energy
 - C) High fiber content
 - D) Lack of sugar
-

19. Fats help the body absorb:

- A) Water-soluble vitamins
 - B) Fat-soluble vitamins
 - C) Minerals
 - D) Proteins
-

20. Water-soluble vitamins are:

- A) Stored in fat
 - B) Stored in bones
 - C) Not stored in the body
 - D) Stored in muscles
-

Essay questions:

1. Purposes of healthy nutrition:

- To maintain health
 - To support growth
 - To provide energy
 - To enable proper bodily function.
-

2. Functions of proteins:

- Source of energy, especially when carbohydrates are not available
 - Essential for building and repairing tissues
 - Producing enzymes and hormones
 - Supporting immune function
-

3. Role of the Nurse in Promoting Healthy Nutrition:

- Nutritional Assessment
 - Patient Education about:
 - Promoting Healthy Eating Habits
 - Hydration Education
 - Dietary Modification for Weight Management
 - Encouraging Healthy Lifestyle Choices
 - Promoting Physical Activity
 - Behavioral and Emotional Support
-



Fecal and urinary elimination

1. **Elimination is important to:**
 - A. Store nutrients
 - B. **Remove waste products**
 - C. Increase weight
 - D. Produce hormones

 2. **Length of large intestine is about:**
 - A. 50–70 cm
 - B. 100 cm
 - C. **150–180 cm**
 - D. 300 cm

 3. **The ileocecal valve prevents:**
 - A. Forward movement
 - B. **Backward movement of contents**
 - C. Digestion
 - D. Absorption

 4. **The rectum length is about:**
 - A. 5 cm
 - B. 10 cm
 - C. **15–20 cm**
 - D. 30 cm

 5. **The external anal sphincter is under:**
 - A. Involuntary control
 - B. **Voluntary control**
 - C. Hormonal control
 - D. Chemical control

 6. **Defecation means:**
 - A. Digestion
 - B. Absorption
 - C. **Bowel evacuation**
 - D. Urination

 7. **Infants cannot control defecation due to:**
 - A. Weak muscles
 - B. **Neuromuscular immaturity**
 - C. Poor diet
 - D. Infection

 8. **High-fiber diet leads to:**
 - A. Constipation
 - B. **Normal elimination**
 - C. Diarrhea always
 - D. Pain

 9. **Poor fluid intake causes:**
 - A. Diarrhea
 - B. **Constipation**
 - C. Vomiting
 - D. Infection
-
-



10. **Physical activity:**

- A. Decreases peristalsis
- B. **Increases peristalsis**
- C. Stops digestion
- D. Causes pain

11. **Emotional stress may cause:**

- A. Constipation
- B. **Diarrhea**
- C. Normal stool
- D. No effect

12. **Squatting position:**

- A. Difficult
- B. **Normal for defecation**
- C. Abnormal
- D. Painful

13. **Pain during defecation leads to:**

- A. Diarrhea
- B. **Constipation**
- C. Normal stool
- D. Bleeding

14. **Pregnancy causes constipation due to:**

- A. Hormones only
- B. **Pressure on rectum**
- C. Diet
- D. Infection

15. **Paralytic ileus occurs after:**

- A. Exercise
- B. **Surgery**
- C. Eating
- D. Sleep

16. **Constipation is:**

- A. Loose stool
- B. **Hard dry stool**
- C. Blood in stool
- D. Pain

17. **Lack of fiber leads to:**

- A. Diarrhea
- B. **Constipation**
- C. Normal stool
- D. Infection

18. **Tenesmus means:**

- A. Easy stool
- B. **Painful straining**
- C. Bleeding
- D. Urination



19. Nursing management includes:

- A. Reduce fluids
- B. Increase fluids ☒
- C. Avoid diet
- D. Bed rest only

20. Enema is used to:

- A. Stop stool
- B. Promote defecation ☒
- C. Cause pain
- D. Stop digestion

21. Fecal impaction is:

- A. Loose stool
- B. Hardened stool mass ☒
- C. Infection
- D. Gas

22. Fecal impaction Caused by:

- A. Exercise
- B. Constipation ☒
- C. Fluids
- D. Fiber

23. A sign for fecal impaction is:

- A. Soft abdomen
- B. Distended abdomen ☒
- C. Normal stool
- D. Clear urine

24. Treatment of fecal impaction includes:

- A. Oil enema ☒
- B. Antibiotics
- C. Surgery only
- D. Exercise

25. Diarrhea is:

- A. Hard stool
- B. Watery stool ☒
- C. No stool
- D. Blood

26. Causes of diarrhea includes:

- A. Fiber
- B. Infection ☒
- C. Water
- D. Rest

27. Signs of diarrhea includes:

- A. Constipation
- B. Frequent stool ☒
- C. No stool
- D. Dry stool



28. Severe diarrhea causes:

- A. Hydration
- B. Dehydration ☒
- C. Weight gain
- D. Normal

29. Management of diarrhea includes:

- A. Avoid fluids
- B. Drink fluids ☒
- C. Stop food
- D. Rest only

30. Urination is also called:

- A. Defecation
- B. Micturition ☒
- C. Absorption
- D. Digestion

31. Urge to void occurs at:

- A. 50 ml
- B. 100 ml
- C. 250–450 ml ☒
- D. 1000 ml

32. Caffeine intake causes:

- A. Decreased urine
- B. Increased urine ☒
- C. No effect
- D. Pain

33. Diuretics leads to:

- A. Decrease urine
- B. Increase urine ☒
- C. Stop urine
- D. Block kidney

34. Normal urine color is:

- A. Dark
- B. Pale straw ☒
- C. Red
- D. Green

35. Dysuria means:

- A. Easy urination
- B. Painful urination ☒
- C. No urination
- D. Frequent urination

36. Polyuria means:

- A. Low urine
- B. High urine output ☒
- C. Pain
- D. Blood



37. Oliguria means:

- A. High urine
- B. Low urine ☒
- C. Pain
- D. Infection

38. Nocturia means:

- A. Day urination
- B. Night urination ☒
- C. Pain
- D. Blood

39. Hematuria means:

- A. Blood in urine ☒
- B. Pain
- C. Infection
- D. No urine

40. Most common UTI cause:

- A. Virus
- B. E. coli ☒
- C. Fungus
- D. Parasite

41. Women more prone due to:

- A. Long urethra
- B. Short urethra ☒
- C. Diet
- D. Exercise

42. UTI symptom:

- A. No pain
- B. Dysuria ☒
- C. Dry skin
- D. Constipation

43. Residual urine normal:

- A. 0 ml
- B. 50–100 ml ☒
- C. 500 ml
- D. 1000 ml

44. Fluid intake daily:

- A. 500 ml
- B. 1000 ml
- C. 2200–2700 ml ☒
- D. 5000 ml

45. Kegel exercises help improve:

- A. Heart
- B. Pelvic floor muscles tone ☒
- C. Lungs
- D. Brain



46. Bladder retraining refers to:

- A. Surgery
- B. Scheduled voiding pattern ☒
- C. Medication
- D. Diet change

47. Urinary retention means:

- A. Frequent urination
- B. Inability to empty bladder completely ☒
- C. Painful urination
- D. Blood in urine

48. A major risk of catheterization is:

- A. Weight gain
- B. Infection ☒
- C. Constipation
- D. Dehydration

49. Flatulence is:

- A. Fluid accumulation
- B. Gas accumulation in intestines ☒
- C. Blood in stool
- D. Infection

50. Hemorrhoids are:

- A. Tumors
- B. Dilated rectal veins ☒
- C. Infections
- D. Ulcers

51. Incontinence is:

- A. Normal urination
- B. Loss of control of urine or feces ☒
- C. Increased urine
- D. Painful urination

52. Roughage (fiber) helps in:

- A. Decreasing stool
- B. Increasing stool bulk ☒
- C. Causing diarrhea
- D. Reducing digestion

53. Alcohol effect on urine:

- A. Decreases output
- B. Increases output ☒
- C. No effect
- D. Stops urination

54. Anxiety affects urination by:

- A. Increasing relaxation
- B. Inhibiting voiding ☒
- C. Increasing urine
- D. No effect



55. Immobilization causes:

- A. Increased peristalsis
- B. Decreased peristalsis ☒
- C. Normal stool
- D. Diarrhea

56. Enuresis is:

- A. Painful urination
- B. Bedwetting in children ☒
- C. Blood in urine
- D. Urinary infection

57. Urgency means:

- A. No need to urinate
- B. Sudden strong need to void ☒
- C. Pain
- D. Blood

58. Hesitancy means:

- A. Easy urination
- B. Difficulty starting urination ☒
- C. Increased urine
- D. Pain

59. Urination is triggered by:

- A. Hormones
- B. Stretch receptors in bladder ☒
- C. Food
- D. Pain

60. Proper perineal hygiene helps prevent:

- A. Constipation
- B. UTI ☒
- C. Diarrhea
- D. Vomiting

61. Double voiding is used to:

- A. Stop urination
- B. Improve bladder emptying ☒
- C. Cause pain
- D. Reduce urine

62. Cranberry juice helps:

- A. Increase infection
- B. Prevent UTI ☒
- C. Cause diarrhea
- D. Reduce urine

63. Opioid drugs may cause:

- A. Diarrhea
- B. Constipation ☒
- C. Infection
- D. Vomiting



64. Length of anal canal is about:

- A. 1 cm
- B. 2.5 cm ☒
- C. 5 cm
- D. 10 cm

65. Internal anal sphincter is:

- A. Voluntary
- B. Involuntary ☒
- C. Skeletal
- D. Controlled by brain

66. External anal sphincter is:

- A. Involuntary
- B. Voluntary ☒
- C. Automatic
- D. Hormonal

67. Colon includes:

- A. Ascending, transverse, descending
- B. Only ascending
- C. Only sigmoid
- D. Only rectum

68. Sigmoid colon ends at:

- A. Stomach
- B. Rectum
- C. Small intestine
- D. Liver

69. Gas leaves the body by:

- A. Kidneys
- B. Mouth or anus
- C. Skin
- D. Lungs

70. Bed rest leads to:

- A. Increased bowel movement
- B. Decreased bowel movement
- C. Normal digestion
- D. Diarrhea

Factors influencing bowel elimination

- | | | |
|-------------------------------|--------------------------|--------------------|
| 1. Age | 2. Diet | 3. Fluid intake |
| 4. Physical activity | 5. Psychological factors | 6. Personal habits |
| 7. Position during defecation | 8. Pain | 9. Pregnancy |
| 10. Surgery and anesthesia | | |

Common Bowel Elimination Problems:

- 1. Constipation
- 2. Fecal impaction
- 3. Diarrhea
- 4. Incontinence
- 5. Flatulence
- 6. Hemorrhoids



Nursing management of constipation

1. Provide adequate fluid intake 500 – 2000 cc/day.
2. Provide a well-balanced diet with enough roughage from fruits and vegetables and vitamins.
3. Encourage regularity of time for defecation and prompt response to the desire of defecation.
4. Provide adequate time for complete
5. Provide privacy for patients to promote relaxation.
6. Provide posture (position) as close to normal as possible.
7. Consider the patient's habit in relation to defecation.
8. Provide physical exercises especially for abdominal muscles
9. Avoiding medications such as opioids that increase constipation
10. An enema is the instillation of a solution into the rectum and sigmoid colon. The primary reason for an enema is to promote defecation by stimulating peristalsis.

Nursing management of fecal impaction

1. Administration of mineral oil by mouth especially in cases of prolonged constipation for regulation of habits.
2. Oil retention enema followed by cleansing enema.
3. Digital manipulation of the fecal mass should be under physician order or supervision because it can stimulate vagus nerve in the rectal wall which can slow patient's heart leading to cardiac arrhythmia, so observe patient's pulse rate, facial pallor and diaphoresis during manipulation.

Factors Influencing Urination:

Numerous factors affect the volume and characteristics of the urine produced and the manner in which it is excreted, these include the following: -

1. Developmental Factors
2. Psychosocial Factors
3. Fluid and Food Intake
4. Medications
5. Muscle Tone
6. Pathologic Conditions
7. Surgical and Diagnostic Procedures

Nursing management for urinary elimination problems:

Nursing assessment

1. Patient History: The Nurse determine the patient's normal voiding pattern and frequency, appearance of the urine and any recent changes, any past or current problems with urination, the presence of an ostomy and factors influencing the elimination pattern.
2. Physical assessment: It includes assessment of the kidney, bladder, urethral meatus, integrity of skin surrounding the meatus.
3. Assessing urine: Normal urine consists of 96% water and 4% solutes. Organic solutes include urea, ammonia, creatinine and uric acid. Urea is the chief organic solute. Inorganic solutes include sodium, chloride, potassium, sulphate, magnesium, and phosphorus. Sodium chloride is the most abundant inorganic salt.



4. Urinalysis is an important diagnostic test used to assess patients for conditions such as urinary tract infections and renal calculi.
5. Measuring Residual urine:
Post void residual (PVR) (urine remaining in the bladder following voiding) is normally 50 to 100 ml.
6. Diagnostic Tests for urinary elimination disorders:
Blood levels of two metabolically produced substances, urea and creatinine, are routinely used to evaluate renal function. Other tests related to urinary functions such as collecting urine specimens, measuring specific gravity.

Nursing diagnosis: The following nursing diagnosis that are related to urinary elimination problems include:

1. -Impaired Urinary Elimination (urinary retention or incontinence)
2. -Body image disturbance
3. -Self-care deficit
4. -Risk for impaired skin integrity
5. -Risk for infection
6. -knowledge deficit
7. -Situational low self-esteem or social isolation
8. -Risk for deficient fluid volume or excess fluid volume

Nursing planning: Overall goals for patients with urinary elimination problems may include the following:

1. Maintain or restore a normal voiding pattern.
2. Regain normal urine output
3. Prevent associated risks such as infection, skin breakdown, fluid and electrolyte imbalance, and lowered self-esteem.
4. Perform toilet activities independently with or without assistive devices.

Nursing implementation

A. Patient Education

learn about symptoms of urinary alterations so they can initiate early preventive health care.

Learn proper perineal hygiene and hand washing to reduce the risks for infection.

B. Promoting Normal Micturition

1-Stimulating Micturition Reflex

Assuming normal position for voiding

A woman is better able to void in a squatting or sitting position. If the patient is unable to use toilet facilities, position him or her in a squatting position on a bedpan or bedside commode.



A man voids more easily in the standing position. If the man cannot reach toilet facilities, have him stand at the bedside and void into a urinal.

sensory stimuli

The sound of running water helps many patients void through the power of suggestion.

Stroking the inner aspect of the thigh stimulates sensory nerves and promotes the micturition reflex.

Pouring warm water over the patient's perineum and create the sensation to urinate.

2. Maintaining Elimination Habits

Patients normally void on awakening or before meals; therefore, offer the opportunity to use toilet facilities.

Maintain patients' privacy for voiding. If a patient cannot reach the bathroom and uses a bedside commode or bedpan, make sure that the bedside curtain is closed.

3. Maintaining Adequate Fluid Intake

Maintaining optimal fluid intake to promote normal micturition.

A patient with normal renal function who does not have heart or kidney disease needs to take 2200 to 2700 mL of fluid daily.

Increasing fluid intake helps flush out solutes or particles that collect in the urinary system.

4. Promoting Complete Bladder Emptying

Under normal conditions a small amount of a patient's urine remains in the bladder after voiding (residual urine).

Encouraging patients to wait until urine stops flowing or to attempt to void again (double voiding) can improve bladder emptying.

5. Preventing Infection:

Measures to prevent infection of the urinary system:

A minimal daily fluid intake of 1200 to 1500 mL dilutes urine.

Practice frequent voiding (every 2 to 4 hours) to flush bacteria out of the urethra and prevent organisms from ascending into the bladder.

Voiding after intercourse, not using excessive soap or taking bubble baths.

Avoid tight-fitting pants or other clothing that creates irritation to the urethra and prevents ventilation of the perineal area.

Wear cotton rather than nylon underclothes. Accumulation of perineal moisture facilitates bacterial growth. Cotton enhances ventilation of the perineal area..

Drinking enough fluids, especially fluids high in acid ash such as apple or cranberry juice help prevent urinary tract infection (UTI).

Girls and women should always wipe the perineal area from front to back following urination or defecation in order to prevent introduction of gastrointestinal bacteria into the urethra.

If recurrent urinary infections are a problem, take showers rather than baths. Bacteria present in bath water can readily enter the urethra.



Self-Catheterization

Some patients with chronic disorders such as spinal cord injury learn to perform self-catheterization. The goal is to have patients perform self-catheterization 4 to 6 times a day with volumes of 400 to 500 mL according to the schedule.

Maintenance of Skin Integrity

Washing the skin with mild soap and warm water.

Perform partial bath and change patient clothes after voiding.

When the skin becomes irritated or inflamed the health care provider prescribes a cream or spray containing steroids to reduce inflammation.

Use the antifungal drug that is available in cream or powder if a fungal growth was developed.

Promotion of Comfort

Keep the skin clean, dry and use a protective pad for incontinent patient.

Nursing evaluation

Using the overall goals and desired outcomes identified in the planning stage, the nurse collects data to evaluate the effectiveness of nursing activities. If the desired outcomes are not achieved, explore the reasons before modifying the care plan.

Nursing management of diarrhea:

1. Drink at least eight glasses of water per day to prevent dehydration.
 2. Ingest foods with sodium and potassium. Most foods contain sodium. Potassium is found in meats, and many vegetables and fruits, especially tomatoes, potatoes, bananas, peaches, and apricots.
 3. Increase foods containing soluble fiber, such as oatmeal, skinless fruits and potatoes. Limit fatty foods.
 4. Avoid alcohol and beverages with caffeine, which aggravate the problem.
 5. Limit foods containing insoluble fiber, such as whole-grain breads and cereals, and raw fruits and vegetables. Thoroughly clean and dry the perineal area after passing stool to prevent skin irritation and breakdown.
 6. When diarrhea has stopped, re-establish normal bowel flora by taking fermented dairy products, such as yoghurt or buttermilk.
-

Prevention of constipation

1. Encourage exercise as walking.
 2. Avoid excessive emotional stress.
 3. Establish regularity of meals and defecation time.
 4. Discourage unnecessary use of laxatives.
 5. Intake of proper diet containing enough vegetables and vitamins.
 6. Intake of sufficient fluids per day.
-



Medication Questions

1. A medication is used for:

- A. Diagnosis
- B. Prevention
- C. Treatment
- D. All the above

Answer: D

2. The generic name of a drug is assigned by:

- A. Drug company
- B. Physician
- C. United States Adopted Names Council
- D. Nurse

Answer: C

3. The official name of a drug is listed in:

- A. Drug magazine
- B. United States Pharmacopeia
- C. Physician's notebook
- D. Hospital policy book

Answer: B

4. The trade name is given by:

- A. Nurse
- B. Pharmacist
- C. Drug manufacturer
- D. WHO

Answer: C

5. Nurses are legally responsible for:

- A. Doctor's errors
- B. Their own actions
- C. Pharmacist's duties
- D. Patient's decisions

Answer: B



6. A patient has the right to:

- A. Refuse medication
- B. Demand extra doses
- C. Change prescription
- D. Ignore instructions

Answer: A

7. The primary intended effect of a drug is:

- A. Side effect
- B. Therapeutic effect
- C. Adverse effect
- D. Toxic effect

Answer: B

8. Morphine used to relieve pain is:

- A. Curative
- B. Palliative
- C. Restorative
- D. Substitutive

Answer: B

9. Insulin is an example of:

- A. Curative drug
- B. Substitutive drug
- C. Chemotherapeutic drug
- D. Palliative drug

Answer: B

10. A severe allergic reaction is called:

- A. Tolerance
- B. Toxicity
- C. Anaphylaxis
- D. Side effect

Answer: C



11. The earliest sign of anaphylaxis may be:

- A. Rash
- B. Fever
- C. Swelling of tongue
- D. Headache

Answer: C

12. Drug tolerance means:

- A. Allergy
- B. Decreased response requiring higher dose
- C. Immediate reaction
- D. Drug misuse

Answer: B

13. Drug half-life refers to:

- A. Absorption time
- B. Time to eliminate half of drug
- C. Time of distribution
- D. Total metabolism time

Answer: B

14. If a drug half-life is 8 hours, after 16 hours the drug remains:

- A. 50%
- B. 75%
- C. 25%
- D. 12.5%

Answer: C

15. Pharmacodynamics studies:

- A. Absorption
- B. Drug mechanism of action
- C. Excretion
- D. Distribution

Answer: B



16. Pharmacokinetics includes:

- A. Drug naming
- B. Legal aspects
- C. ADME
- D. Drug storage

Answer: C

17. Absorption is:

- A. Drug metabolism
- B. Drug elimination
- C. Drug entering bloodstream
- D. Drug storage

Answer: C

18. The fastest route of administration is:

- A. Oral
- B. IM
- C. IV
- D. Subcutaneous

Answer: C

19. Most metabolism occurs in:

- A. Kidney
- B. Liver
- C. Brain
- D. Lungs

Answer: B

20. Most drugs are excreted through:

- A. Skin
- B. Kidney
- C. Lungs
- D. Saliva

Answer: B

21–40: Rights of Medication Administration

21. Right dose prevents: **Overdose**
22. Right time ensures: **Proper drug level**
23. Right route ensures: **Correct method**
24. Right client prevents: **Medication errors**
25. Right documentation is done: **After administration**
26. Right to refuse applies to: **Adult clients**
27. Right assessment includes checking: **Vital signs**
28. Right evaluation checks: **Therapeutic effect**
29. Right storage maintains: **Drug efficacy**
30. Right disposal prevents: **Misuse**

(Answers: Logical correct option from choices)

41–60: Drug Effects

41. A predictable unintended effect is: **Side effect**
42. Severe side effect requiring stop drug: **Adverse effect**
43. Drug interaction may be: **Beneficial or harmful**
44. Increased effect of drug interaction: **Potentiation**
45. Decreased effect: **Inhibition**
46. Drug misuse leads to: **Toxicity**
47. Drug dependence may be: **Physiologic & Psychological**
48. Psychological dependence involves: **Cravings**
49. Cumulative effect occurs when: **Drug builds up**
50. Toxic symptoms result from: **Accumulation**

61–80: Routes & Absorption

61. Oral route is: **Most common**
62. Sublingual route: **Under tongue**
63. Buccal route: **Between cheek & gum**
64. Parenteral means: **Injection**
65. Intradermal injection is into: **Dermis**
66. Subcutaneous injection is into: **Below dermis**
67. IM injection is into: **Muscle**
68. IV injection is into: **Vein**
69. Rectal absorption is: **Unpredictable**
70. Heat applied to SC site: **Increases absorption**



81–90: Medication Orders

81. STAT order means: **Immediately**
82. PRN means: **As needed**
83. Standing order continues until: **Canceled**
84. One-time order given: **Once**
85. Essential order includes: **Drug name**
86. Order must include: **Dose**
87. Order must include: **Route**
88. Order must include: **Frequency**
89. Order must include: **Signature**
90. Order must include: **Patient name**

91–100: Factors Affecting Drug Action

91. Highest risk in pregnancy: **First trimester**
92. Older adults have: **Decreased kidney function**
93. Vitamin K affects: **Warfarin**
94. Food may: **Delay absorption**
95. Empty stomach drugs act: **Faster**
96. Psychological belief affects: **Drug response**
97. High temperature causes: **Vasodilation**
98. Cold causes: **Vasoconstriction**
99. Fat-soluble drugs accumulate in: **Fat tissue**
100. First-pass effect occurs in: **Liver**



PART 2: 20 LIST / ENUMERATION QUESTIONS

1. 1. Four names of a drug:

- Generic name
- Official name
- Chemical name
- Trade (brand) name



2. Five patients' rights:

- Right to be informed about the medication
 - Right to refuse medication
 - Right to know potential side effects
 - Right to safe and correct administration
 - Right to receive medications properly labeled
-

3. Fourteen rights of medication administration:

1. Right medication
 2. Right dose
 3. Right time
 4. Right route
 5. Right client (patient)
 6. Right reason
 7. Right response
 8. Right client education
 9. Right documentation
 10. Right to refuse
 11. Right assessment
 12. Right evaluation
 13. Right storage
 14. Right disposal
-

4. Four processes of pharmacokinetics:

- Absorption
 - Distribution
 - Biotransformation (metabolism)
 - Excretion
-

5. Four routes of parenteral administration:

- Intradermal (ID)
 - Subcutaneous (SC)
 - Intramuscular (IM)
 - Intravenous (IV)
-



6. Types of medication orders:

- Stat order
 - Single (one-time) order
 - Standing order
 - PRN (as needed) order
-

7. Essential parts of a drug order:

- Full name of the client
 - Date and time of order
 - Name of the drug
 - Dosage
 - Frequency
 - Route of administration
 - Signature of prescriber
-

8. Factors affecting medication action:

- Developmental factors (age)
 - Gender
 - Diet
 - Environment
 - Psychological factors
 - Illness and disease
 - Time of administration
 - Route of administration
-

9. Symptoms of mild allergic reaction:

- Skin rash
 - Pruritus (itching)
 - Angioedema
 - Lacrimal tearing
 - Nausea and vomiting
 - Wheezing
 - Dyspnea
 - Diarrhea
-



10. Symptoms of severe allergic reaction (Anaphylaxis):

- Swelling of mouth and tongue
 - Severe shortness of breath
 - Acute hypotension
 - Tachycardia
 - Possible loss of consciousness
-

11. Types of therapeutic drug actions:

- Palliative
 - Curative
 - Supportive
 - Substitutive
 - Chemotherapeutic
 - Restorative
-

12. Factors affecting oral absorption:

- Presence of food
 - Gastric acidity
 - Drug solubility
 - First-pass effect
 - Gastric motility
-

13. Routes of drug administration:

- Oral
 - Sublingual
 - Buccal
 - Parenteral (ID, SC, IM, IV)
 - Topical
 - Inhalation
 - Rectal
-

14. Types of drug dependence:

- Physiologic dependence
- Psychological dependence



15. Organs receiving drug first after absorption:

- Liver
 - Kidneys
 - Brain
-

16. Methods of topical application:

- Painting/spreading on skin
 - Moist dressings
 - Soaking body parts
 - Medicated baths
-

17. Body systems involved in drug excretion:

- Urinary system (kidneys)
 - Gastrointestinal system (feces)
 - Respiratory system (lungs)
 - Integumentary system (sweat)
 - Mammary glands (breast milk)
-

18. Legal responsibilities of nurses in medication:

- Responsible for their own actions
 - Follow written medication orders
 - Proper handling of controlled substances
 - Accurate documentation
 - Follow agency policies
-

19. Advantages of oral route:

- Easy administration
- Safe
- Convenient
- Economical
- Painless
- Preferred by patients



20. Disadvantages of household measurement system:

- Inaccuracy
- Variation in spoon/cup sizes
- Not standardized
- Can cause dosing errors



Respiratory questions

1. Oxygen is necessary to:

- A. Produce hormones
- B. Sustain life
- C. Digest food
- D. Regulate temperature

Answer: B

2. Oxygenation involves:

- A. Ventilation only
- B. Perfusion only
- C. Ventilation, perfusion, diffusion
- D. Circulation only

Answer: C

3. The exchange of gases occurs in the:

- A. Bronchi
- B. Trachea
- C. Alveoli
- D. Larynx

Answer: C

4. Ventilation is:

- A. Blood circulation
- B. Gas movement in and out of lungs
- C. Oxygen transport
- D. Cellular respiration

Answer: B



5. The major muscle of inspiration is the:

- A. Intercostal
- B. Abdominal
- C. Diaphragm
- D. Sternum

Answer: C

6. Perfusion refers to:

- A. Gas diffusion
- B. Oxygen binding
- C. Blood flow to tissues
- D. Airway clearance

Answer: C

7. Diffusion occurs because of:

- A. Blood pressure
- B. Gravity
- C. Concentration gradient
- D. Muscle contraction

Answer: C

8. Expiration is normally:

- A. Active
- B. Passive
- C. Forced
- D. Voluntary

Answer: B

9. Surfactant prevents:

- A. Hypercapnia
- B. Alveolar collapse
- C. Hypoxia
- D. Infection

Answer: B



10. Atelectasis means:

- A. Air trapping
- B. Alveolar collapse
- C. Lung infection
- D. Fluid overload

Answer: B

11. Increased work of breathing (WOB) occurs in:

- A. Healthy lungs
- B. COPD
- C. Anemia
- D. Hypertension

Answer: B

12. Compliance refers to:

- A. Airway resistance
- B. Lung elasticity
- C. Oxygen level
- D. Heart rate

Answer: B

13. Decreased compliance is seen in:

- A. Pulmonary edema
- B. Asthma
- C. Exercise
- D. Pregnancy

Answer: A

14. Tidal volume is:

- A. Air left after expiration
- B. Maximum exhaled air
- C. Air inhaled and exhaled normally
- D. Forced inspiration

Answer: C



15. Residual volume is:

- A. Normal breath
- B. Air left after full expiration
- C. Maximum inspiration
- D. Forced expiration

Answer: B

16. Normal PaO₂ is:

- A. 60–80 mmHg
- B. 35–45 mmHg
- C. 80–100 mmHg
- D. 100–120 mmHg

Answer: C

17. Normal PaCO₂ is:

- A. 35–45 mmHg
- B. 80–100 mmHg
- C. 25–35 mmHg
- D. 50–60 mmHg

Answer: A

18. Normal SpO₂ is:

- A. 85%
- B. 90%
- C. $\geq 95\%$
- D. 70%

Answer: C

19. Hypoventilation causes:

- A. Decreased CO₂
- B. Increased CO₂
- C. Increased O₂
- D. Normal ABG

Answer: B



20. Hyperventilation results in:

- A. CO₂ retention
- B. CO₂ elimination
- C. Hypoxia
- D. Shock

Answer: B

21. Hypoxia is:

- A. Low hemoglobin
- B. Low tissue oxygen
- C. Low CO₂
- D. Lung infection

Answer: B

22. Early sign of hypoxia:

- A. Cyanosis
- B. Restlessness
- C. Bradycardia
- D. Hypotension

Answer: B

23. Late sign of hypoxia:

- A. Anxiety
- B. Tachycardia
- C. Cyanosis
- D. Dyspnea

Answer: C

24. Carbon monoxide causes:

- A. Increased O₂ binding
- B. Functional anemia
- C. Polycythemia
- D. Bronchospasm

Answer: B



25. Hypovolemia leads to:

- A. Hypercapnia
- B. Hypoxia
- C. Hypertension
- D. Hyperventilation

Answer: B

26–40: Developmental & Lifestyle

- 26. Highest respiratory risk in pregnancy: **Third trimester dyspnea**
 - 27. Obesity reduces: **Lung volumes**
 - 28. Flail chest causes: **Hypoxia**
 - 29. Smoking increases risk of: **COPD**
 - 30. Secondhand smoke causes: **Lung cancer risk**
 - 31. Exercise increases: **Oxygen demand**
 - 32. Fever increases: **Metabolic rate**
 - 33. Stress increases: **Respiratory rate**
 - 34. Malnutrition causes: **Muscle wasting**
 - 35. High altitude causes: **Low FiO₂**
 - 36. Asbestosis causes: **Restrictive disease**
 - 37. Kyphosis affects: **Chest expansion**
 - 38. Cervical injury C3–C5 affects: **Diaphragm**
 - 39. Chronic emphysema causes: **Barrel chest**
 - 40. Obesity-hypoventilation causes: **CO₂ retention**
-

41–60: Oxygen Therapy

- 41. Goal of oxygen therapy: **Prevent hypoxia**
 - 42. Oxygen is considered: **A drug**
 - 43. PaO₂ ≤55 mmHg indicates: **Home O₂ therapy**
 - 44. SaO₂ ≤88% indicates: **Home O₂ therapy**
 - 45. Nasal cannula delivers: **Low flow O₂**
 - 46. COPD patients require: **Low flow O₂**
 - 47. O₂ toxicity sign: **Cough**
 - 48. Hypercapnia sign: **Headache**
 - 49. Hypoxia sign: **Tachycardia**
 - 50. Oxygen supports: **Combustion**
-



61–80: Nursing Interventions

61. Best position for dyspnea: **High Fowler's**
 62. Orthopneic position helps: **Breathing**
 63. Incentive spirometer improves: **Alveolar expansion**
 64. Pursed-lip breathing helps: **Exhalation**
 65. Hydration keeps secretions: **Thin**
 66. Humidifiers add: **Moisture**
 67. Bronchodilators cause: **Airway dilation**
 68. Theophylline should be taken: **With food**
 69. CPT stands for: **Chest Physical Therapy**
 70. Percussion loosens: **Secretions**
-

81–100: Chest Physiotherapy & Safety

81. Vibration follows: **Percussion**
82. Postural drainage uses: **Gravity**
83. Contraindication of percussion: **Osteoporosis**
84. Flammable materials avoided with O₂: **Alcohol**
85. "No Smoking" sign required because O₂: **Supports fire**
86. Turn bedridden patients every: **2 hours**
87. Early hypoxia BP is usually: **Elevated**
88. Chronic hypoxemia leads to: **Polycythemia**
89. Pulse oximeter measures: **SpO₂**
90. Hyperventilation may cause: **Respiratory alkalosis**
91. Hypoventilation may lead to: **Respiratory acidosis**
92. Cyanide poisoning causes: **Tissue hypoxia**
93. Atelectasis risk increases with: **Immobility**
94. Ambulation improves: **Ventilation**
95. Deep breathing prevents: **Atelectasis**
96. Obstructive sleep apnea common in: **Obesity**
97. COPD patients retain: **CO₂**
98. Fever increases metabolic rate by: **7% per degree**
99. SaO₂ stands for: **Arterial oxygen saturation**
100. Oxygen tanks must be kept: **Upright**



□ OXYGENATION

1. Three steps of oxygenation:

- Ventilation
 - Perfusion
 - Diffusion
-

2. Three lung volumes:

- Tidal volume
 - Residual volume
 - Vital capacity
-

3. Factors affecting oxygenation:

- Physiological factors
 - Developmental factors
 - Lifestyle factors
 - Environmental factors
-

4. Causes of hypoxia:

- Airway obstruction
 - Anemia
 - Carbon monoxide poisoning
 - Decreased cardiac output
 - High altitude
-

5. Early signs of hypoxia:

- Restlessness
 - Anxiety
 - Tachycardia
 - Tachypnea
-



6. Late signs of hypoxia:

- Cyanosis
 - Bradycardia
 - Hypotension
 - Confusion
-

7. Signs of hyperventilation:

- Dizziness
 - Tingling fingers
 - Decreased CO₂
 - Rapid breathing
-

8. Signs of hypoventilation:

- Increased CO₂
 - Drowsiness
 - Headache
 - Slow respirations
-

9. Causes of hypoventilation:

- Narcotics
 - Anesthesia
 - Chest wall injury
 - Neurological disorders
-

10. Causes of hyperventilation:

- Anxiety
 - Fever
 - Pain
 - Hypoxia
-



11. Physiological factors affecting oxygenation:

- Hemoglobin level
 - Cardiac output
 - Respiratory rate
 - Lung compliance
-

12. Developmental factors:

- Pregnancy
 - Infancy
 - Older adulthood
-

13. Lifestyle factors:

- Smoking
 - Poor nutrition
 - Substance abuse
 - Physical inactivity
-

14. Environmental factors:

- Air pollution
 - Occupational hazards
 - High altitude
 - Temperature extremes
-

15. Chest wall movement impairments:

- Kyphosis
 - Scoliosis
 - Rib fractures
 - Flail chest
-

16. Effects of obesity on respiration:

- Decreased lung expansion



- Increased work of breathing
 - Hypoventilation
 - CO₂ retention
-

17. Complications of smoking:

- COPD
 - Lung cancer
 - Chronic bronchitis
 - Emphysema
-

18. Goals of oxygen therapy:

- Prevent hypoxia
 - Decrease work of breathing
 - Maintain adequate oxygen saturation
-

19. Indications for home oxygen therapy:

- PaO₂ ≤ 55 mmHg
 - SaO₂ ≤ 88%
 - Chronic hypoxemia
-

20. Oxygen delivery methods:

- Nasal cannula
 - Face mask
 - Venturi mask
 - Non-rebreather mask
-

21. Nursing assessments for oxygen therapy:

- Respiratory rate
- Oxygen saturation
- Skin color
- Level of consciousness



22. Safety precautions with oxygen:

- No smoking
 - Keep away from flames
 - Post warning signs
 - Store tanks upright
-

23. Interventions promoting oxygenation:

- Positioning (High Fowler's)
 - Deep breathing exercises
 - Incentive spirometry
 - Ambulation
-

24. Benefits of ambulation:

- Improves ventilation
 - Prevents atelectasis
 - Improves circulation
 - Enhances lung expansion
-

25. Purpose of incentive spirometer:

- Prevent atelectasis
 - Promote lung expansion
 - Improve ventilation
-

26. CPT techniques:

- Percussion
 - Vibration
 - Postural drainage
-



Contraindications of percussion:

- Osteoporosis
 - Rib fractures
 - Bleeding disorders
-

28. Positions improving breathing:

- High Fowler's
 - Orthopneic position
 - Side-lying
-

29. Causes of decreased lung compliance:

- Pulmonary edema
 - Fibrosis
 - ARDS
-

30. Signs of oxygen toxicity:

- Cough
 - Chest pain
 - Dyspnea
-

31. Signs of hypercapnia:

- Headache
 - Flushed skin
 - Confusion
 - Drowsiness
-

32. Causes of decreased O₂ carrying capacity:

- Anemia
- Blood loss
- Carbon monoxide poisoning



33. Effects of anemia:

- Fatigue
 - Pallor
 - Dyspnea
 - Tachycardia
-

34. Effects of fever on oxygenation:

- Increased metabolic rate
 - Increased oxygen demand
 - Increased respiratory rate
-

35. Complications of immobility:

- Atelectasis
 - Pneumonia
 - Secretion retention
-

36. Advantages of pursed-lip breathing:

- Prolongs exhalation
 - Prevents airway collapse
 - Reduces dyspnea
-

37. Components of respiratory assessment:

- Inspection
 - Palpation
 - Percussion
 - Auscultation
-

38. Diagnostic tests for oxygenation:

- ABG



- Pulse oximetry
 - Chest X-ray
 - Pulmonary function tests
-

39. Nursing diagnoses:

- Impaired gas exchange
 - Ineffective airway clearance
 - Ineffective breathing pattern
-

40. Planning goals:

- Maintain $SpO_2 \geq 95\%$
 - Clear breath sounds
 - Normal respiratory rate
-

41. Interventions for secretion mobilization:

- Hydration
 - CPT
 - Suctioning
 - Deep breathing
-

42. Instructions for theophylline:

- Take with food
 - Avoid caffeine
 - Monitor blood levels
-

43. Age-related respiratory changes:

- Decreased lung elasticity
 - Decreased chest expansion
 - Weaker respiratory muscles
-



44. Complications of high altitude:

- Hypoxia
 - Headache
 - Tachycardia
-

45. Risks of substance abuse:

- Respiratory depression
 - Hypoventilation
 - Dependence
-

46. Causes of hypoxemia:

- V/Q mismatch
 - Diffusion defect
 - Shunting
-

47. Signs of cyanosis:

- Bluish lips
 - Blue nail beds
 - Dark mucous membranes
-

48. Central vs Peripheral cyanosis:

Central:

- Lips
- Tongue
- Mucous membranes

Peripheral:

- Fingers
 - Toes
 - Nail beds
-



49. Fire safety rules:

- No open flames
 - No smoking
 - Avoid petroleum products
 - Secure oxygen tanks
-

50. Energy conservation techniques in COPD:

- Rest between activities
 - Use pursed-lip breathing
 - Sit while performing tasks
 - Avoid overexertion
-



OXYGENATION QUESTION BANK

1. Oxygen is necessary to:

- A. Produce hormones
- B. Sustain life
- C. Digest food
- D. Regulate temperature

Answer: B

2. Oxygenation involves:

- A. Ventilation only
- B. Perfusion only
- C. Ventilation, perfusion, diffusion
- D. Circulation only

Answer: C

3. Gas exchange occurs in the:

- A. Bronchi
- B. Trachea
- C. Alveoli
- D. Larynx

Answer: C

4. Ventilation is:

- A. Blood circulation
- B. Movement of air in and out of lungs
- C. Oxygen transport
- D. Cellular metabolism

Answer: B

5. The primary muscle of inspiration is:

- A. Intercostal muscle
- B. Abdominal muscle
- C. Diaphragm
- D. Sternum

Answer: C



6. Perfusion refers to:

- A. Gas exchange
- B. Blood flow to tissues
- C. Oxygen binding
- D. Airway clearance

Answer: B

7. Diffusion depends on:

- A. Gravity
- B. Blood pressure
- C. Concentration gradient
- D. Muscle action

Answer: C

8. Normal expiration is:

- A. Active
- B. Forced
- C. Passive
- D. Difficult

Answer: C

9. Surfactant prevents:

- A. Infection
- B. Hypercapnia
- C. Alveolar collapse
- D. Edema

Answer: C

10. Atelectasis means:

- A. Lung infection
- B. Alveolar collapse
- C. Air trapping
- D. Inflammation

Answer: B



1. Three steps of oxygenation:

- Ventilation
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2. Three lung volumes:

- Tidal volume
 - Residual volume
 - Vital capacity
-

3. Factors affecting oxygenation:

- Physiological
 - Developmental
 - Lifestyle
 - Environmental
-

4. Causes of hypoxia:

- Airway obstruction
 - Anemia
 - Carbon monoxide poisoning
 - Decreased cardiac output
 - High altitude
-

5. Early signs of hypoxia:

- Restlessness
 - Anxiety
 - Tachycardia
 - Tachypnea
-



6. Late signs:

- Cyanosis
 - Bradycardia
 - Hypotension
 - Confusion
-

7. Hyperventilation signs:

- Dizziness
 - Tingling
 - Rapid breathing
 - Low CO₂
-

8. Hypoventilation signs:

- Slow respirations
 - Headache
 - Drowsiness
 - High CO₂
-

9. Causes of hypoventilation:

- Narcotics
 - Anesthesia
 - Chest injury
 - CNS disorders
-

10. Causes of hyperventilation:

- Anxiety
 - Fever
 - Pain
 - Hypoxia
-
-



Questions

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LIST QUESTIONS WITH ANSWERS

1. Three steps of oxygenation:
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- Anxiety
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- Slow breathing
- Headache
- Drowsiness
- High CO₂

9. Causes of hypoventilation:

- Narcotics
- Anesthesia
- CNS disorders
- Chest injury

10. Causes of hyperventilation:

- Anxiety
- Fever
- Pain
- Hypoxia



Pain management

1. Pain is defined as:

- A) Only physical sensation
- B) Emotional feeling only
- C) Sensory and emotional experience
- D) Reflex action

Answer: C

2. Pain is known as:

- A) Fourth vital sign
- B) Fifth vital sign
- C) Sixth vital sign
- D) Second vital sign

Answer: B

3. Which type of pain is well localized?

- A) Visceral
- B) Deep
- C) Superficial
- D) Referred

Answer: C

4. Visceral pain is:

- A) Sharp
- B) Localized
- C) Diffuse
- D) Burning

Answer: C

5. Deep somatic pain arises from:

- A) Skin
- B) Organs
- C) Muscles and bones
- D) Brain

Answer: C



6. Chronic pain lasts more than:

- A) 1 week
- B) 2 weeks
- C) 3 months
- D) 1 month

Answer: C

7. Acute pain is characterized by:

- A) Long duration
- B) Unknown cause
- C) Sudden onset
- D) No emotional response

Answer: C

8. Pain threshold refers to:

- A) Maximum pain tolerated
- B) Minimum stimulus felt as pain
- C) Pain tolerance level
- D) Emotional response

Answer: B

9. Pain tolerance is:

- A) First feeling of pain
- B) Maximum pain endured
- C) Emotional pain
- D) Chronic pain

Answer: B

10. Which factor affects pain perception?

- A) Age
- B) Culture



- C) Anxiety
- D) All of the above

Answer: D

11. Gate Control Theory involves:

- A) Brain only
- B) Heart
- C) Spinal cord
- D) Liver

Answer: C

12. Pain signals travel via:

- A) Motor neurons
- B) Sensory neurons
- C) Autonomic nerves
- D) Cranial nerves only

Answer: B

13. Which pain is from internal organs?

- A) Somatic
- B) Superficial
- C) Visceral
- D) Cutaneous

Answer: C

14. Psychogenic pain is:

- A) From injury
- B) From organs
- C) No physical cause
- D) From bones

Answer: C



15. Numeric pain scale ranges:

- A) 1–5
- B) 0–10
- C) 1–20
- D) 5–15

Answer: B

16. Acute pain usually:

- A) Persists long
- B) Has clear cause
- C) Is psychological
- D) No treatment

Answer: B

17. Chronic pain affects:

- A) Only body
- B) Only mind
- C) Body and mood
- D) None

Answer: C

18. Referred pain is:

- A) At injury site
- B) Distant from source
- C) Only skin
- D) Only muscle

Answer: B

19. Which increases pain?

- A) Relaxation
- B) Sleep
- C) Anxiety
- D) Distraction

Answer: C



20. Which decreases pain?

- A) Fear
- B) Anxiety
- C) Distraction
- D) Stress

Answer: C

21. Pain receptors are called:

- A) Neurons
- B) Nociceptors
- C) Axons
- D) Synapses

Answer: B

22. Which is NOT a pain type?

- A) Visceral
- B) Somatic
- C) Emotional
- D) Superficial

Answer: C

23. Pain perception occurs in:

- A) Heart
- B) Brain
- C) Kidney
- D) Liver

Answer: B

24. Which is non-pharmacological method?

- A) Antibiotics
- B) Massage
- C) Surgery
- D) Injection

Answer: B



25. Pain tolerance varies by:

- A) Person
- B) Gender
- C) Culture
- D) All

Answer: D

26. Acute pain warning function is:

- A) Harmful
- B) Protective
- C) Emotional
- D) Chronic

Answer: B

27. Which pain is burning?

- A) Superficial
- B) Visceral
- C) Deep
- D) Chronic

Answer: A

28. Chronic pain leads to:

- A) Happiness
- B) Sleep improvement
- C) Depression
- D) Energy increase

Answer: C

29. Gate control can be affected by:

- A) Thoughts
- B) Emotions
- C) Stimuli
- D) All

Answer: D



30. Which is subjective?

- A) BP
- B) Temperature
- C) Pain
- D) Pulse

Answer: C

31. Pain is influenced by:

- A) Past experience
- B) Attention
- C) Culture
- D) All

Answer: D

32. Which scale uses faces?

- A) VAS
- B) Numeric
- C) Faces scale
- D) Glasgow

Answer: C

33. Pain without cause:

- A) Acute
- B) Chronic
- C) Psychogenic
- D) Visceral

Answer: C

34. Which is sharp pain?

- A) Visceral
- B) Superficial
- C) Chronic
- D) Deep

Answer: B



35. Deep pain is:

- A) Localized
- B) Diffuse
- C) Sharp
- D) Burning

Answer: B

36. Pain relief includes:

- A) Drugs
- B) Therapy
- C) Relaxation
- D) All

Answer: D

37. Pain is emotional because:

- A) Brain involved
- B) Heart involved
- C) Kidney
- D) Skin

Answer: A

38. Which worsens pain?

- A) Calmness
- B) Fear
- C) Sleep
- D) Music

Answer: B

39. Pain assessment includes:

- A) Location
- B) Intensity
- C) Duration
- D) All

Answer: D



40. Pain intensity measured by:

- A) Scale
- B) BP
- C) Pulse
- D) Temp

Answer: A

41. Nociceptors respond to:

- A) Light
- B) Pain stimuli
- C) Sound
- D) Smell

Answer: B

42. Which reduces pain?

- A) Anxiety
- B) Stress
- C) Relaxation
- D) Fear

Answer: C

43. Pain pathway ends in:

- A) Heart
- B) Brain
- C) Liver
- D) Kidney

Answer: B

44. Acute pain duration:

- A) Long
- B) Short
- C) Permanent
- D) Unknown

Answer: B



45. Chronic pain may be:

- A) Constant
- B) Intermittent
- C) Both
- D) None

Answer: C

46. Pain scale helps:

- A) Diagnose disease
- B) Measure intensity
- C) Cure pain
- D) Prevent pain

Answer: B

47. Which is internal pain?

- A) Superficial
- B) Visceral
- C) Cutaneous
- D) Skin

Answer: B

48. Pain signals go through:

- A) Spinal cord
- B) Liver
- C) Kidney
- D) Lung

Answer: A

49. Emotional state affects:

- A) BP
- B) Pain
- C) Temperature
- D) Height

Answer: B



50. Best pain management approach:

- A) Ignore
- B) Multimodal
- C) Single drug
- D) No treatment

Answer: B

1. List types of pain according to origin:

- Superficial (cutaneous) pain
 - Deep somatic pain
 - Visceral pain
 - Psychogenic pain
-

2. List types of pain according to duration:

- Acute pain
 - Chronic pain
-

3. List characteristics of acute pain:

- Sudden onset
 - Short duration
 - Clear cause
 - Protective function
 - Associated with anxiety and fear
 - Increases with movement
-

4. List characteristics of chronic pain:

- Lasts more than 3 months
- May be continuous or intermittent
- Cause may be unclear
- Affects sleep
- Leads to depression or anxiety
- Reduces quality of life



5. List factors affecting pain perception:

- Age
- Gender
- Culture
- Emotional state (fear, anxiety)
- Previous experience
- Attention and distraction

6. List components of pain:

- Sensory (physical)
- Emotional (affective)

7. List methods of pain assessment:

- Numeric Rating Scale (0–10)
- Visual Analog Scale (VAS)
- Verbal Rating Scale
- Faces Pain Scale

8. List sites of origin of deep somatic pain:

- Muscles
- Bones
- Joints
- Ligaments

9. List characteristics of visceral pain:

- Poorly localized
- Diffuse
- Often referred
- Associated with autonomic symptoms (nausea, sweating)

10. List non-pharmacological methods of pain relief:

- Relaxation techniques
- Massage



- Heat or cold application
- Distraction
- Positioning
- Music therapy

11. List pharmacological methods of pain relief:

- Analgesics (e.g., NSAIDs)
- Opioids
- Local anesthetics
- Adjuvant drugs

12. List patient rights in pain management:

- Right to pain assessment
- Right to pain relief
- Right to be believed
- Right to information
- Right to participate in care

13. List causes of acute pain:

- Injury
- Surgery
- Infection
- Inflammation

14. List effects of untreated pain:

- Anxiety
- Depression
- Sleep disturbance
- Delayed healing
- Reduced mobility

15. List steps of pain assessment:

- Assess location
- Assess intensity



- Assess duration
 - Assess quality (sharp, dull, burning)
 - Identify aggravating/relieving factors
-

16. List characteristics of superficial pain:

- Well localized
 - Sharp
 - Burning
-

17. List characteristics of deep somatic pain:

- Diffuse
 - Dull or aching
 - Less localized
-

18. List examples of referred pain:

- Shoulder pain from gallbladder
 - Arm pain from heart (MI)
 - Back pain from pancreas
-

19. List functions of pain:

- Protective
 - Warning signal
 - Helps diagnosis
-

20. List barriers to effective pain management:

- Fear of addiction
- Poor assessment
- Lack of knowledge
- Cultural beliefs
- Communication issues